Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

Version: 0.9

HMIP/02

Hospital Major Incident Plan
YSBYTY GWYNEDD

Date to be reviewed:	October 2024	**No of pages:**	182
Author(s):	Helen Granton	**Author(s) title:**	Interim Lead Manager - EC
Responsible Department / Director:	Ffion Johnstone, IHC Director (West) Dafydd Pleming, Interim Site Manager		
Approved by:	BCU Civil Contingencies Group / IHC Management Group		
Date approved:	12 January 2021		
Date activated (live):			
Updated:	November 2024 – YG Major Incident Review Group		
Review:	November 2025		

Documents to be read alongside this policy:	Major Emergency Plan

Purpose of Issue/Description of current changes:

This is the operational Hospital Major Incident Plan for the Ysbyty Gwynedd site.

All Departments have reviewed their Action Cards and changes have been made to the majority of cards.

First operational:	July 2012				
Previously reviewed:	2016	2019	2020	2023	2024
Changes made yes/no:	yes	yes	yes	yes	yes

PROPRIETARY INFORMATION
This document contains proprietary information belonging to the Betsi Cadwaladr University Health Board. Do not produce all or any part of this document without written

Introduction

The threat posed by modern environmental emergencies, terrorism, infectious diseases or major industrial accidents are likely to be the greatest challenge faced by the NHS in recent times. With a potential impact of increased demand, a reduced or compromised workforce and the challenges associated with remodelling the service to meet the needs of patients. Emergencies of this scale and nature would in themselves be unprecedented in nature and scale and thus require a whole systems approach to the problem.

Following a major transition programme Betsi Cadwaladr University Health Board created a Major Emergency Plan that sets out how the BCUHB will react to such an emergency and how it will contribute to the overarching and co-ordinated multi-agency response. Furthermore, the plan will identify safe, workable systems that can be delivered to ensure that the public continues to receive a professional health service irrespective of whether their needs are as a result of the emergency or an underlying medical condition.

The aim of the following plan is to outline how each District General Hospital site will respond to and manage a co-ordinated response to deal with a potential event.

Not all aspects of the plan will be relevant to everyone, however, you should ensure that you read the sections pertinent to you and your department (including any departmental Major Incident Plans), so you are prepared should the Health Board be faced with the challenge of a Major Incident.

Document Control

Policy	Procedure	Protocol	Guideline
Yes	Yes	No	No

Classification of document:	Corporate
Area for Circulation:	Health Board Wide
Reference number:	HMIP/02
Version Number:	0.9
Original/previous Ref number:	0.4
Author Name and Job Title:	Ysbyty Gwynedd Major Incident Review Group
Responsible Officer:	Mrs Carol Shillabeer, Chief Executive
Details of Lead/Responsible:	Imran Devji Interim Chief Operating Officer
Group/ Committee:	Civil Contingencies Group
Consulted Via:	BCU Civil Contingencies Group/Head of Staff
Ratified by:	Civil Contingencies Group/HMT
Date issued:	May 2014
Review details:	November 2024

Title	Page

Introduction	1
Document Control	2
Part 1 – Definition of an Emergency	4 - 7
Part 2 – Triggers and Alert Levels	7 - 11
Part 3 – Role of the Acute Hospitals	11
Part 4 – Co-ordination and Multi Agency Integration	12
Part 5 – Patient Flow Diagrams	13
Part 6 – Action Cards	14 - 187

Part 1

Definitions and Trigger Levels

Definition of an Emergency

BCUHB defines a 'major emergency' or a 'major incident' as:

"Any occurrence that presents a serious threat to the health of the community, disruption to the service or causes, or is likely to cause, such numbers or types of casualties as to require special arrangements to be implemented by a hospital(s), Ambulance Trust or primary care organisation."

BCUHB also recognises the wider Civil Contingencies Act definition as:

"An event or a situation which threatens serious damage to human welfare in a place in the UK, to the environment of a place in the UK, or war or terrorism which threatens serious damage to the security of the UK."

BCUHB also recognises certain events have the potential to rapidly overwhelm the local capacity to respond, even with the implementation of major emergency plans.

Therefore this plan takes account of mass casualty incidents and defines them as:

"*A disastrous single or simultaneous event(s) or other circumstances where the normal major incident response of several NHS organisations must be augmented by extraordinary measures in order to maintain an effective, suitable and sustainable response*."

Declaration of an Emergency

BCUHB can collectively declare a major emergency or individual hospitals and community or primary care responders can declare a major incident when their own facilities and/or resources are overwhelmed.

BCUHB recognises that other partner agencies may declare a major emergency which may not meet the above health definition and that the health definition might not result in a major emergency for others. BCUHB will activate an appropriate level of response to support other civil protection partners should they declare a major emergency.

Prioritisation Categories of Patients

P1 (Red) immediate intervention within one hour – life threatening
P2 (Yellow) admission and early intervention within four hours
P3 (Green) can wait for treatment – usually the walking wounded

All casualties will be triaged and labelled on site in accordance with the agreed casualty priority sorting code/labelling system

Hospital Declaration

In the majority of cases the alerting message will come from the Welsh Ambulance Services NHS Trust and will be received by the Hospital Switchboard. The alert may also come via the Welsh Government, internally from a hospital, community or primary care area, any other NHS organisation such as Public Health Wales or via any of the emergency or statutory services.

If a call is received directly by the Emergency Department, then the ED staff member receiving the call must immediately inform Switchboard.

Scale of Emergency

"Beyond a Major Incident" (DOH 2004) sets guidance and policy to assist the NHS to plan for a major incident of very serious proportions involving potentially large numbers of casualties. This Guidance defines the scale of a major incident in the NHS.

Whilst many major incidents are triggered by 'big bang' incidents such as traffic accidents, explosions etc., there are other potential circumstances where a major incident is triggered by a rising tide or non-acute traumatic event, e.g., infectious disease outbreak, power cuts, covert radiation leakage. In such cases, the ambulance services may be involved but may not be the natural alerting organisation.

The levels of incidents for which NHS organisations are required to develop emergency preparedness arrangements are:

Major – for example multi vehicle motorway crashes. More patients will be dealt with, probably faster and with fewer resources than usual but it is possible to maintain the usual levels of service.

Mass – much larger scale events affecting potentially hundreds rather than tens of people, possibly involving the closure or evacuation of a major facility or persistent disruption over many days. These will require a collective response by several or many neighbouring authorities.

Catastrophic – events that severely disrupt health and social care and other functions, for example mass casualties, power, water etc. and that exceed even collective local capability within the NHS.

There are also events which occur on a national scale e.g. fuel strikes terrorist attacks, pandemic or multiple events that require the collective capability of the NHS nationally.

A major incident may arise in a variety of ways:

- **Big Bang:** a serious transport accident, explosion or a series of smaller incidents.
- **Rising tide:** a developing infectious disease epidemic or a capacity/staffing crisis pandemic.
- **Cloud on the horizon:** a serious threat such as a major chemical or nuclear release developing elsewhere and needing preparatory action.
- **Headline news:** public or media alarm about a personal threat.
- **Internal incidents:** fire, breakdown of utilities, major equipment failure, hospital acquired infections; violent crime
- **Deliberate release of chemical, biological or nuclear materials**
- **Mass casualties**

Triggers

The exact trigger point for initiating BCUHB involvement in a major emergency is difficult to define and will depend on the nature and extent of the incident. Therefore, the following situations may trigger a major emergency response:

- A significant incident that threatens to overwhelm the resources of one or more hospitals, community or primary care area

- A significant incident that requires coordination of more than one hospital, community or primary care area

- An incident that crosses into the area covered by another Welsh Health Board or equivalent English health structure

- An incident where mutual aid is requested, such as on a regional or national basis

- An incident that requires the attendance of health representation at the North Wales Strategic Coordination Group

- An incident that requires coordination of health resources and the opening of communication links with the Welsh Government Emergency Co-ordination Centre (ECCW), any other Government Office or with Public Health Wales (PHW)

- A significant internal incident within any hospital, community or primary care whereby the daily running of the service is adversely affected necessitating special arrangements to be instigated. Where this incident is realised the specific Business Continuity Plan must also be activated

- A significant incident that requires media coordination, particularly with partner organisations

Activation of the BCUHB Major Emergency Plan

The decision to activate the Major Emergency Plan lies with the on-call Director (Gold) who will consult with the On-Call Silver, Bronze, other appropriate staff groups and our partner agencies such as Welsh Ambulance Services Trust or North Wales Police.

A major incident alert may come from a variety of sources, including:

- The Welsh Ambulance Services NHS Trust (WAST)
- Internally from a hospital, community or primary care area
- Welsh Government
- Any other NHS organisation such as Public Health Wales
- Any of the emergency or statutory services

Part 2

Alert Levels

The normal "Major Incident" declaration arises through the Welsh Ambulance Services NHS Trust Control Room to the designated receiving hospital switchboard. However, incidents might arise that do not affect the WAST and therefore notification may be received through other means (for example North Wales Police) or through an internal cascade.

There are six levels of alert:

1. **Major Incident Standby -** this is when the incident does not require an immediate response, but there is the potential for the incident to escalate and a decision will be made to send out a 'stand by alert' to the Health Board and the incident will be monitored and if necessary a major incident can be declared.

2. **Major Incident Declared –** this is when the incident requires an immediate response and the Health Board major incident plan is activated.

3. **Major Incident Declared - Mass Casualty Incident –** when the threshold of the mass casualty definition has been met (where the number/type of casualties overwhelms the conventional major incident response) and the activation of the Mass Casualty Incident Arrangements for NHS Wales is required.

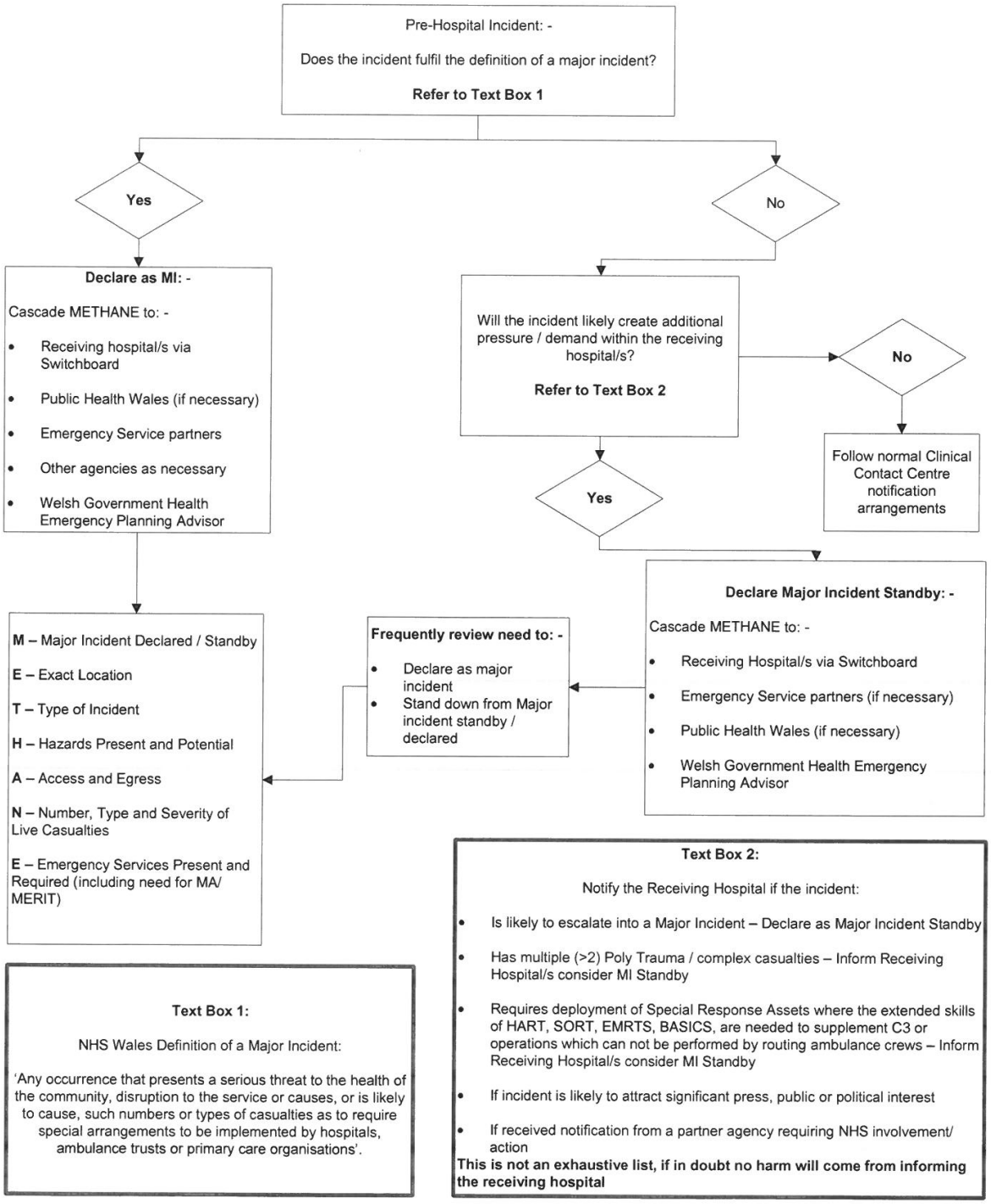
4. **Major Incident Cancelled –** Cancels either the first or second message.

5. **Major Incident – Scene Evacuation Complete -** issued by WAST to confirm all patients have been evacuated from scene and will confirm any that are on route to specific hospitals.

6. **Major Incident Stand down –** notifies us when an incident is over at scene. It is the responsibility of all responding agencies to determine when their organisation should stand down.

Draft final version
Dec 2016
For Review: Dec 2017

Process Map to Support Declaration of a Major Incident / Major Incident Standby



Text Box 1:

NHS Wales Definition of a Major Incident:

'Any occurrence that presents a serious threat to the health of the community, disruption to the service or causes, or is likely to cause, such numbers or types of casualties as to require special arrangements to be implemented by hospitals, ambulance trusts or primary care organisations'.

Text Box 2:

Notify the Receiving Hospital if the incident:

- Is likely to escalate into a Major Incident – Declare as Major Incident Standby
- Has multiple (>2) Poly Trauma / complex casualties – Inform Receiving Hospital/s consider MI Standby
- Requires deployment of Special Response Assets where the extended skills of HART, SORT, EMRTS, BASICS, are needed to supplement C3 or operations which can not be performed by routing ambulance crews – Inform Receiving Hospital/s consider MI Standby
- If incident is likely to attract significant press, public or political interest
- If received notification from a partner agency requiring NHS involvement/ action

This is not an exhaustive list, if in doubt no harm will come from informing the receiving hospital

Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

Communications cascade for NHS response to a Major Incident
Relevant to BCU Health Board and BCU areas of WAST only

Ymddiriedolaeth GIG Gwasanaethau Ambiwlans Cymru Welsh Ambulance Services NHS Trust

Definitions of a Major Incident

NHS
Any occurrence that presents a serious threat to the health of the community, disruption to the service or causes, or is likely to cause, such numbers or types of casualties as to require special arrangements to be implemented by hospitals, ambulance trusts or primary care organisations

CCA
An event or a situation which threatens serious damage to human welfare in a place in the UK, to the environment of a place in the UK, or war or terrorism which threatens serious damage to the security of the UK.

Significant / Critical Incident's
Certain events or situation could lead to additional burden within the NHS such as serious multiple casualty, environmental incidents, mass gatherings, public protest or those with public/political interest.
It is important for WAST and BCU to be able to assess the impact these incidents may have on services.

Hospital Incident
Individual NHS organisations can self-declare a major incident when their own facilities / resources or those of its neighbours are overwhelmed. Such as situations where the day-to-day operation of the hospital is adversely effected due to an internal business disruption creating an unsafe environment.

The Welsh Ambulance Service (WAST) operates a 24/7 Clinical Contact Centre which receives, deploys and co-ordinates all aspects of emergency ambulance response. In support of this function WAST employ a Bronze, Silver, NILO and Gold command hierarchy that is used to manage the activity of the Service. Furthermore, WAST is the gate keeper to activating the wider NHS response to a major incidents, which includes that of the Health Board and Public Health Service.

Betsi Cadwaladr Health Board (BCU) provides Scheduled (planned) and Unscheduled (Emergency) care to the population of North Wales. It delivers services through three acute district general hospitals, community hospital and nursing teams, mental health services as well as commissioning GP service and directly providing GP Out of Hours Services. The health board operate an out of hours on-call arrangement with a Operational (Bronze) for each DGH, a pan north Wales Tactical (Silver) and Strategic (Gold). In addition mental health operate a standalone on-call.
In response to major incidents BCU may choose to open a Hospital Control Centre (HCC) at one or all receiving hospitals, which will be responsible for coordination of patient flow at that site and be managed by the Site Director or Operational (Bronze) On-call. In addition to aid coordination across the whole health economy BCU may choose to open a Health Emergency Control Centre (HECC) which will be managed by the Tactical (Silver) On-call. In the event of a multi-agency Tactical Coordination Group (TCG) BCU Silver will decide whether they are best placed at the TCG or the HECC. The Strategic (Gold) on-call would attend the Strategic Coordination Centre (SCG) if established.
The HCC and HECC are only activated in response to an on-going incident.

CONTACTS

WAST:

CCC Non urgent Tel:	0300 1239224	**Urgent /emergency:**	999
CCC Duty Control Manager:	01248 682001	**Email:**	amb_NorthShiftmanagers@wales.nhs
On-call Managers:	via CCC	**Email:** (Silvers only)	amb_North_Region_Silver@wales.nhs.uk
NILO:	0300 1232312	**Email:**	amb_wast-nilo@wales.nhs.uk

BCU: **Ysbyty Gwynedd:** 01248 384384 **Ysbyty Glan Clwyd** 01745 583910 **Wrexham Maelor** 01978 291100
YG HCC: 01248 385390 **YGC HCC:** 01745 584422 **WM HCC:** 01978 725904
BCU HECC: 01248 681976 / 689062 **Email:** BCU.HECCSilver@wales.nhs.uk (available only when activated)

ACTIONS

Pre-Hospital Major Incident or Standby:
WAST will: -
- Identify and declare Major Incident / Major Incident Standby
- Indentify receiving and supporting hospitals
- Cascade METHANE to receiving and supporting hospital via switchboards

BCU will: **-**
- Review the METHANE report and trigger an appropriate hospital response

Significant health incidents:
WAST will: **-**
- Identify health based incidents which could lead to extreme pressure within the health system but below that of a major incident.
- Notify the receiving hospital via the Clinical Site Manager (CSM) and / or Emergency Department

BCU will:
- Review the information provided and take appropriate actions to manage the situation

Critical Incidents: If in any doubt contact can be made with the WAST NILO
Partner agencies should: **-**
- Determine whether the incident is likely to require ambulance intervention and if so notify the WAST Silver via CCC
- Determine whether the incident is likely to require wider NHS intervention and if so notify the BCU Silver via the nearest Hospital Switchboard

M	Major Incident Declared or Standby?
E	Exact Location
T	Type of Incident
H	Hazards present or potential
A	Access and Egress—safe routes
N	Number, type, severity of casualties
E	Emergency services present and required?

OFFICIAL SENSITIVE

VRS: 1.0 23.01.2016

Major Incident Cascade

On notification of a Major Incident from WAST, the designated Hospital Switchboard will be provided with a METHANE report. In hours they will then notify the Clinical Site Manager (Bleep 100), ED Consultant and Hospital Director. Out of hours they will inform the Clinical Site Manager (Bleep 100), ED Consultant and Hospital Director / Bronze on Call.

The Site Manager, Hospital Director / Bronze on call and ED Consultant will review the METHANE report and advise Switchboard on the next course of action.

This could be to either:

- Do nothing further at this stage
- Declare a Major Incident Standby for the hospital site
- Declare a Major Incident for the hospital site

On instruction from the Site Manager, the declaration will be instigated from the Switchboard via the internal bleep system / phone cascade to the wider hospital teams. All relevant departments will be informed using the phrase “Major Incident Declared” or “Major Incident Standby”.

The on call Silver and on call Gold will be informed and briefed at this time. The on call Silver should challenge this declaration if they do not feel this course of action is appropriate.

As part of the alerting procedure, the on-call Director (Gold) will confirm the immediate course of action to be taken by BCUHB and the command and control arrangements to be implemented. The command and control arrangements should be proportionate and based on advice from the Chief Executive, Executive Director of Planning and Performance, WG Health Emergency Planning Adviser, and in discussion with the On-call Silver and Hospital Managing Director/Bronze on call where the incident has an initial local focus.

When deciding organisations to be alerted, consideration needs to be given to informing non-health organisations.

To enable BCUHB to exercise appropriate judgement about the necessity to take overall strategic command of an incident, it is important that BCUHB is notified of ALL major incidents of whatever scale occurring within the region.

In and Out of Hours

Once alerted, the on-call Director (Gold) will follow the advice in the **‘On-Call Gold’ Action Card 1** in Part 6 of this plan. The on-call Director (Gold) will initiate the call out cascade for key staff to support the incident at a gold level and identify a **“Staff Officer”** and **“Loggist”** to attend the SCG (Action cards for these roles are located in part 6). An alternative Director or on-call Silver may be contacted to act as the “Staff Officer” whilst trained Loggists details are stored on the On-Call website.

Furthermore, the on-call Director (Gold) will liaise with the on-call Silver to consider establishing the HECC, and identifying an appropriate member of staff to undertake the role as the Incident Commander (Silver) from this location.

Notifying Welsh Government

In the event of a major emergency it is the responsibility of the Incident Commander (Silver) to notify the WG Health Emergency Planning Advisor / On-Call Officer using the major incident contact details found on the On-Call website.

Part 3

Role of the Acute Hospitals

The key roles and responsibilities of the hospital sites are to:

- Provide the hospital's clinical response
- Receive and triage casualties in the Emergency Department
- Liaise with the WAST, other hospitals and partner agencies to manage the impact of the incident
- Provide support to any designated receiving hospital that is substantially affected and in return, rely on mutual support if required
- Maintain communications with relatives and friends of existing casualties and those from the incident, the local community, the media and VIP's
- Provide on-site medical care and advice
- Provide counselling advice and support in partnership with other agencies
- Ensure the Health Board continues all of its essential functions and the maintenance, as far as possible, of normal acute health services

Ysbyty Gwynedd, Ysbyty Glan Clwyd and the Wrexham Maelor Hospital are the three 'receiving' hospitals within North Wales in the event of a Major Incident. This means that depending on the geographical area of the incident, any site will receive the first casualties from any Major Incident occurring within North Wales.

If all of the injured patients cannot be treated at one specific site, some will be directed to other hospitals; this will be determined by the Ambulance Service according to the nature and location of the incident.

Whilst a Major Incident is in progress, in addition to being designated the receiving hospital, the site will still receive emergency medical and surgical admissions from its catchment area. It may therefore be necessary to divert such admissions to neighbouring hospitals.

Part 4

Co-ordination and Multi Agency Integration

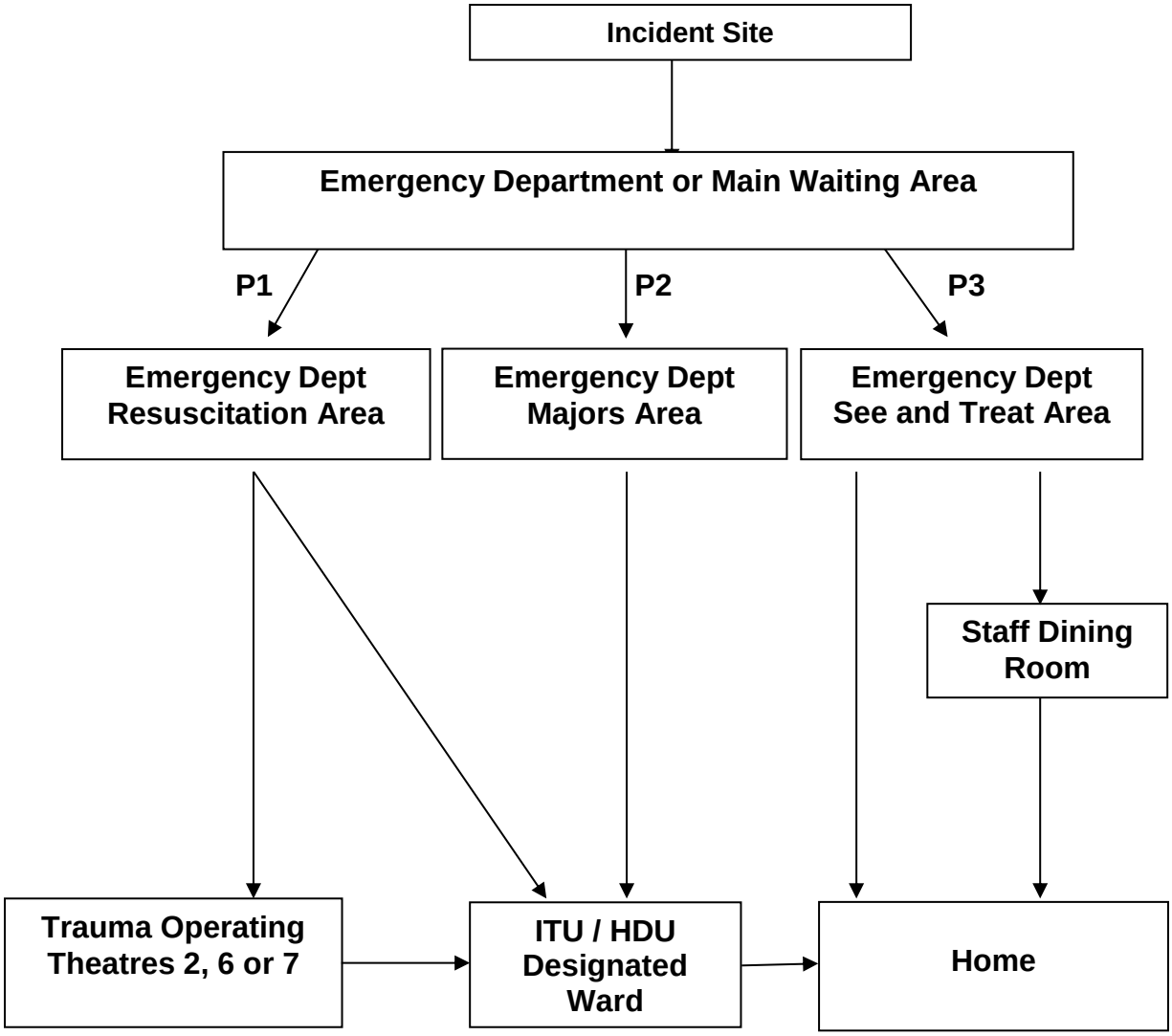
The command and control of major emergencies by BCUHB will be in accordance with the Strategic (Gold), Tactical (Silver) and Operational (Bronze) Command Structure as follows below:

Command	Definition	Action
Gold	Strategic Command of the Incident	Provide the strategic management and co-ordination of BCUHB resources during the emergency by ensuring secondary, community and primary care service delivery for both the incident and for normal operational delivery. Establishes and communicates policy and determines priorities for the tactical command to implement. Allocate resources to ensure appropriate tactical and operational response. Establish strategies for the return to normality. Communicates with the Welsh Government and the public. Actively or passively represents the BCUHB at the Strategic Co-ordination Centre.
Silver	Tactical Command of the Incident	At BCUHB level provide the tactical management and co-ordination of resources during the emergency by directing secondary, community and primary care services. Implement the strategic policy and priorities. Prioritise the allocation of resources. Liaison with other agencies and ensuring effective co-ordination of the tactical response. Direct Bronze commander(s) according to the requirements of the incident. Co-ordinate multiple hospital responses and deliver a primary care response.
Bronze	Operational Command of Response	Operationally manage all resources in line with strategic direction and established priorities to provide an acute response.

Part 5

Major Incident Procedure – Patient Flow

West



Throughout a Major Incident, the routine emergency work of the hospital must be accommodated as far as possible.

Part 6

Hospital Action Cards

	Page No
Switchboard	16
Clinical Site Manager / CSM A	20
Matron / CSM (B)	22
Duty Administrator	24
Hospital Incident Commander (HIC)	26
Medical Incident Commander (MIC)	30
Nursing Incident Commander (NIC)	32
Hospital Command Centre (HCC)	34
Duty Administrator Loggist	36
Hospital Incident Commander (HIC) Loggist	38
Medical Incident Commander (MIC) Loggist	40
Nursing Incident Commander (NIC) Loggist	42
Emergency Department (ED) Consultant	44
Nurse in Charge, Emergency Department	46
Emergency Department (ED) Receptionist	48
Estates Facilities Cell	50
Information Management Cell	52
Non Acute Services CelIs	54
Staffing Cell	56
Support Services Cell	58
Media Liaison Office	60
Ambulance Liaison Officer	62
Police Documentation Team	68
Discharge Support Team	72
Pharmacy – In Hours	74
Pharmacy – Out of Hours	76
Catering Department	78
Facilities Department	80
Estates Department	82
CSSD / HSDU	84
Junior Doctor / Registrar	86
Consultant Orthopaedic Surgeon On-call	88
Consultant Surgeon On-call	90
Consultant Paediatrician On-call	92
Blood Science and Transfusion	94
Mortuary Manager	96
Chaplaincy	98
Relatives Officer	100
Health Records Officer	102
Locality Matron - Community Hospitals	104
Locality Matron - District Nursing	106
Radiology (Cards 1 - 16)	108 - 139
Anaesthetic Department (Cards 1 - 9)	140 - 163
Intensive Care (Cards 1 - 6)	164 - 171
Theatres (Cards 1 - 9)	172 - 187
Appendix 1 - Handover Procedure	188
Appendix 1 - JESIP - Joint Decision Model	190

BLANK SHEET

This card is for Switchboard Staff

YG MAJOR INCIDENT ACTION CARD: Switchboard

Role:

The Switchboard will usually receive first notification of a Major Incident from the Ambulance Service, however, a Major Incident could be declared by a member of the senior medical/nursing team in the Emergency Department and/or the Clinical Site Manager.

The words “Major Incident, standby” or “Major Incident declared” should be used. Standby notification will be given when a potential Major Incident has been notified to the Ambulance Service but they have not yet reached the incident site to assess its scale, or when an incident is thought likely to escalate to a scale that would require activation of the Major Incident Procedure within the Hospital.

	Action	**Completed By: Date & Time**
1	**Call Received from Ambulance** The telephonist will receive an automated call from Ambulance to the dedicated phone in Switchboard. Complete the blank METHANE report and once completed, PRESS 1 to confirm receipt or PRESS 2 to replay the message. (The message will replay or you will receive multiple calls until the message is confirmed by pressing 1). • Identity of caller • **M**ajor Incident • **E**xact location of incident • **T**ype of incident • **H**azards present at the site • **A**ccess to the incident site • Approximate **N**umber of casualties • **E**mergency Services currently at the site Verification email will be sent from Ambulance Control to the switchboard email group – please acknowledge receipt of email and print off a copy of the METHANE Report for the CSM.	
2	**In hours, the Hospital Switchboard will immediately notify the Clinical Site Manager (Bleep 100), Emergency Department Consultant (Bleep 999 / X841198) and Hospital Director (X840452). Out of Hours, the Hospital Switchboard will contact the Clinical Site Manager (Bleep 100), Emergency Department Consultant (Bleep 999 / X841198), Hospital Director (see On-call list) and Bronze On-call (see rota).**	

	The information contained in the METHANE Report should be provided and these key individuals will decide how to proceed with the Major Incident or indeed if this is a Major Incident for the Site / Health Board. **Out of Hours – Switchboard On-call Supervisor should be informed.**	
3	**Call Received from Emergency Department** Take the caller's name and number ask them to leave the line free and tell them you will call them back immediately to verify the call. Contact needs to be made immediately to the ED Consultant (Bleep 999 / X841198), CSM (Bleep 100) and Operational Manager – a discussion needs to take place. If the decision is made to declare Major Incident Standby/Major Incident Declared then key individuals should be notified via the Crash/Fire Bleep System (*1 & *3), using the phrase "Major Incident, Standby or Declared". The telephonists should then immediately start to alert key members of staff, according to the call cascades at the end of this document. **There are two lists - one for use during normal working hours, the other for out of hours, weekends and bank holidays.** Staff and departments should be contacted in the order they appear on the list (but if an individual cannot be located quickly the switchboard operator should continue down the list before trying again to contact those individuals who could not be reached the first time). Each individual should be given the same message: **"The Hospital has been placed on Major Incident Standby. Please activate the standby phase of your Major Incident response."**	
4	As each individual is contacted a note should be made on the call-out list of their name and the time they were contacted. It is essential that these people are contacted as quickly as possible - for this reason the Switchboard operators should make all of these calls without answering any incoming calls to the main switchboard. If on-call staff have to be paged, the message should be for them to ring in to line (**03000 850108).** This will enable switchboard staff to identify incoming calls that need to be answered as part of the response. If more than one switchboard operator is present the individual receiving the notification should make these calls whilst the other can continue to deal with incoming calls. The Clinical Site Manager will identify a member of staff to assist switchboard with incoming calls of a complex nature.	

	The Hospital Switchboard need to complete the telephone call cascades.	
5	If the alert is then confirmed (by receiving a "Major Incident Declared" message from Ambulance Control) the call out process should then be followed according to the procedure described in "INCIDENT DECLARED" below. The Switchboard operator should then re-contact those members of staff who have been placed on standby notification, informing them that the alert is declared. If the decision is made to declare Major Incident, then key individuals should be notified via the Crash/Fire Bleep System (*1 & *3) using the phrase "Major Incident Declared". The telephonists should then immediately start to alert key members of staff, according to the call cascades at the end of this document stating: **"The previous notification of Major Incident Standby has been escalated to Major Incident Declared. Please activate the declared phase of your Major Incident response."**	
6	**Major Incident Cancelled** If you receive a message stating that the Major Incident has been cancelled ring Ambulance Control to check the message and then notify key individuals via the crash/fire bleep system (*1 & *3) using the phrase "Major Incident Cancelled" then re-contact those members of staff who have been placed on standby/declared informing them that the Major Incident is cancelled. Please follow the Call Cascade list.	
7	**Major Incident Stand Down** When you receive confirmation from the Hospital Command Centre that the incident has been stood down, notify key individuals via the crash/fire bleep system (*1 & *3) using the phrase "Major Incident Stand Down". Re-contact those members of staff following the Call Cascade list who have been placed on standby/declared stating that the Major Incident has been stood down.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Clinical Site Manager (CSM A)

Role:

- The Clinical Site Manager (A) will initially carry out the role of the Nursing Incident Commander (NIC)
- Immediately contact the Head of Nursing for the Emergency Quadrant Directorate / or relevant Directorate or Deputy or if available a second Clinical Site Manager who can perform the role of Clinical Site Manager (B)
- The Clinical Site Manager (A) will proceed to the Emergency Department to determine the appropriate hospital response having considered the METHANE Report received from WAST/Switchboard
- Hospital Management Team members, will, if available, be able to provide advice following implementation of the cascade

	Action	**Completed By: Date & Time**
1	**Major Incident Standby** • You will have already been notified by Switchboard and will have been asked to proceed to Switchboard to review the METHANE Report with the On Call ED Consultant and the Operational Manager/Bronze on Call. There must be a minimum of 2 people to make the decision on how to proceed. • Following your decision, contact Switchboard and advise them how to proceed – either start call cascade or stand down. • Identify a suitable Directorate Operational Manager to support Switchboard staff and assist with telephone calls of a complex nature – calls should be transferred to the Hospital Command Centre (HCC - Board Room) as soon as it is operational. The Operational Manager will come from the Manager of the Day rotas. • Establish Hospital Command Centre, ensure tables and chairs are in position and install telephone handsets. • Ensure that up to date bed information on bed states across all wards is available.	
2	• Ensure Loggists are contacted to commence recording telephone calls and decisions in the Major Incident Log Books. • When the Lead Nurse Unscheduled Care/Clinical Site Manager (B) arrives at the HCC, Clinical Site Manager (A) should proceed to the	

	Emergency Department and follow the remaining actions on this card. • Liaise with the HCC to establish anticipated numbers of patients and the time before arrival from scene to determine whether existing inpatients could be moved to AMU/SAU to enable a dedicated ward to be allocated for the admission of patients. • Support Emergency Department in clearing the Emergency Department by identifying beds for the rapid admission of patients currently within the Emergency Department. • Liaise with the Community Matron of the Day about the discharge and transfer of patients and the resulting bed availability in the Community Area.	
3	**Major Incident Declared** • Ensure that all the steps above at Major Incident Standby have been carried out. • Liaise with the HCC to establish anticipated numbers of patients and the time before arrival from scene to determine whether existing inpatients should be moved to enable AMU/SAU to be allocated for the admission of patients. • At the direction of the HCC, liaise with wards, junior medical staff and Portering staff to relocate existing patients within the Hospital.	
4	**During the Incident** • The CSM (A) will facilitate admission to AMU/SAU once the decision to admit has been made from the Emergency Department. • If necessary the CSM will work with Junior Medical Staff and wards to co-ordinate the transfer of existing inpatients between wards to ensure that appropriate ward accommodation is provided for patients with specific needs. • The CSM (A) will ensure that up to date information on bed states is relayed to the HCC and the Ambulance Liaison Officer for onward transmission to the Medical Incident Officer at the incident site. • If insufficient beds are available for the expected number of patients the CSM must inform the HCC of this. The HCC will decide whether to seek patients for transfer to other hospitals or discharge home. • Throughout the incident the CSM (A) will communicate and liaise closely with Head of Nursing for the Emergency Quadrant Directorate or Deputy / CSM (B) and ensure that up to date information on bed states and patient flow is available in the HCC.	

YG MAJOR INCIDENT ACTION CARD:
Matron / CSM (B)

Role:

The Head of Nursing/CSM (B) will support the Hospital Management Team in coordination of the incident within the Hospital Command Centre.

The Head of Nursing/CSM (B) will do this by assisting the Nurse in Charge, liaising with the CSM in relation to patient flow and ensuring clinical staff resources are sufficient to facilitate patient flow.

If the Head of Nursing Medicine/CSM is not available this role will be carried out by a designated Clinical Site Manager.

	Action	Completed By: Date & Time
1	**Major Incident Standby** The Matron CSM (B) will be contacted by the on-site CSM (or via the Major Incident bleep if during working hours) and will report to the HCC. • Take a handover from the Clinical Site Manager on duty (CSM A) and continue to set up the HCC ensuring tables and chairs are in position and installing telephone handsets. • Start recording all telephone calls in the Major Incident Log Books. • Liaise with the Discharge Liaison Nurses**,** Therapy Team and the area lead for Enhanced Care, Lead Nurse and Community for the West area about the discharge and transfer of patients and the resulting bed availability in the Community Hospitals.	
2	**Major Incident Declared** • Proceed to the Hospital Command Centre. • Take a handover from the Clinical Site Manager on duty (CSM A) and continue to set up the HCC ensuring tables and chairs are in position and installing telephone handsets. • Start recording all telephone calls in the Major Incident Log Books. • Oversee patient flow by ensuring sufficient clinical staffing is available in each of the critical areas receiving patients, that bed capacity is made available to meet the anticipated numbers of patients and that specialities are ready to receive critical patients.	

	• Liaise with the Discharge Support Team about the discharge and transfer of suitable patients and the resulting bed availability. • During a protracted incident the Matron/CSM (B) will need to arrange a roster system for the CSM Team, this will ensure that effective site management and discharge planning can be maintained throughout the duration of the incident.	

YG MAJOR INCIDENT ACTION CARD:
Duty Administrator

Role:

The Duty Administrator will support the Hospital Management Team to co-ordinate the incident within the Hospital Command Centre (HCC)

The Duty Administrator will support the site in setting up the Hospital Command Centre as per illustrations in the Major Incident pack.

	Action	Completed By: Date & Time
1	**Major Incident Standby** The Duty Administrator will be notified of the Major Incident Standby by the HIC/MIC/NIC (during working hours) and will report to the HCC. Out of hours the role will be allocated to the first Loggist who arrives at the HCC. • Take a handover from the Clinical Site Manager on duty (CSM A) and continue to set up the HCC ensuring tables and chairs are in position and installing telephone handsets to support CSM B. • Start recording all telephone calls in the Major Incident Log Books. • Keep an initial log of staff as they arrive and roles that have been allocated.	
2	**Major Incident Declared** • Proceed to the Hospital Command Centre (HCC). • Take a handover from the Clinical Site Manager on duty (CSM A) and continue to set up the HCC ensuring tables and chairs are in position and installing telephone handsets to support CSM B. • Start recording all telephone calls in the Major Incident Log Books. • Keep a running log of staff as they arrive and roles that have been allocated. • Maintain restricted access to the HCC and ensure an accurate log of those that enter and exit the HCC.	

	• During a protracted incident the Duty administrator will need to arrange a roster system to ensure a function role 24/7, this will ensure that effective HCC log is maintained.	

This card is for the Hospital Incident Commander (HIC)

YG MAJOR INCIDENT ACTION CARD:
Hospital Incident Commander (HIC)

Role:

The Hospital Incident Commander (HIC) will be the Managing Director/Hospital Director in hours and initially the Bronze On-call out of hours. They will have responsibility for co-ordinating the hospital's response to the incident – the Bronze on-call can delegate this role if necessary to another member of the on-call team.

The Hospital Incident Commander (HIC) can be contacted on **03000 840490**.

The HIC will be notified by switchboard that the hospital has been put on Major Incident standby. During normal working hours the HIC should proceed to the ED to discuss how to proceed with the incident along with the CSM and the ED Consultant. OOH they should contact the ED to discuss the appropriate hospital response with the ED Consultant and the CSM.

The HIC will co-ordinate the arrangements for the reception of patients and will liaise with supporting hospitals and Silver on-call for the Health Board.

HIC will make arrangements for the treatment of patients and provision of extra staffing.

The CSM (A) will be responsible for contacting the HIC, ED Consultant and the Physician on Call to determine the appropriate hospital response having considered the METHANE Report received from WAST.

	Action	**Completed By: Date & Time**
1	**Major Incident Standby** • Consider the SITREP (METHANE) in terms of the short, medium and long-term consequence.	
2	**Don't instigate the actions below unless Major incident declared.** • Ensure Hospital Command Centre is appropriately staffed and the following are identified and availability is confirmed to attend if necessary: o Hospital Director or HMT representative o Five members of the Acute Directorate/Bronze on Call (In hours, HMT PA's will have contacted five members of the Bronze On Call/ Acute Directorate. Out of hours Bronze on call will be responsible for doing this o Four Loggists	

	• Identify the bed status/staffing for ED, Medical, Surgery and Anaesthetics, Theatres, Critical Care, Admissions Ward, Community Hospitals. • Consider the cancellation of non-urgent elective, inpatient day case and outpatient activity taking advice from the NIC and MIC. Is this an action in the NIC and MIC action cards • Determine staffing status/capacity for: • Imaging • Pathology • Medical • Nursing • Catering • Portering • Domestics • Pharmacy • Medical Records • Operational teams • Ensure that a detailed log is maintained throughout the incident (log books available in HCC).	
3	**Major Incident Declared** • Activate arrangements outlined at Major Incident Standby. • Consider the SITREP (METHANE) in terms of the short, medium and long term consequence. • Task the NIC, supported by the CSM (B), On-Call/Clinical Directors ECS, Medicine, Surgery or Deputies to oversee patient flow, ambulance liaison, patient discharge, outpatients, ED, Surgery, Medicine, Women Services and Children Services. • Task the MIC to liaise with medical colleagues and support effective clinical management. • Task the six members of the Bronze On-call Team, to oversee the following cells: and agree a timeframe on which to report back: **i.** **Information Management Cell –** including media liaison, public enquiries, switchboard, administrative support. **ii.** **Estates/Facilities Cell –** Portering, Hotel Services, Laundry, Traffic, Utilities, Equipment, Security.	

	iii. **Support Services Cell –** Police Documentation Team, Relatives, Medical Records, Pathology, Mortuaries, HSDU/CSSD, Pharmacy and Physiotherapy, IM&T, Radiology, Theatres. iv. **Non Acute Services Cell –** Community Care, Primary Care and Mental Health Liaison , Hospital Social Worker, Voluntary Aid Societies. v. **Staffing Cell –** Nurse Bank, Therapy Support, staff side liaison, Human Resources, appropriate specialities and departments. vi. **Relatives Centre Co-ordinator –** provide support to relatives of casualties involved in the Major Incident. • Action Cards to be issued and members of staff to be briefed. This should be recorded within the log. • Acknowledge and act where appropriate on any site/incident specific plans i.e. decontamination plans. • Ensure hospital communications are implemented i.e. two way radios or other systems used within the hospital. • Admissions Ward to be identified and made ready to receive patients, within the AMU/SAU. • Other medical staff to be deployed appropriately in accordance with action cards. • Silver HECC will advise HCC – which specialist units have been alerted (depending on the incident). i.e. Major Trauma at UHNS, MTC • Co-ordinate the allocation of manpower, equipment, blood products, oxygen etc at the hospital site/major incident site, if requested. • Determine when it is appropriate to stand the hospital down in its entirety or component parts, ensuring that all records are secured and that an appropriate debrief is scheduled to take place at the earliest opportunity. • Write a summary report for the Chief Executive. Consider arrangements for recovery phase. Secure appropriate HCC Runners to HCC for tasking.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Medical Incident Commander (MIC)

Role:

The Medical Incident Commander will be the Anaesthetist / Duty Physician On-call until relieved by the Hospital Site Medical Director (or Deputy) and will have responsibility for co-ordinating the hospital's response to the incident along with the HIC and NIC.

The Medical Incident Commander (MIC) can be contacted on **03000 840492.**

	Action	**Completed By: Date & Time**
1	**Major Incident Standby** The ED Consultant on-call will be contacted by the Clinical Site Manager who, together with the Bronze on-call, will determine the appropriate hospital response having considered the METHANE Report received from WAST.	
2	**Major Incident Declared** • Ensure that a detailed log is maintained throughout the incident • Liaise with the On-call Consultants and NIC within the HCC to ensure the effective deployment and management of medical staff • Co-ordinate the prioritisation of individual patient management with clinical colleagues in the context of competing priorities • Provide a central point of contact for clinical decision making dealing with organisations and agencies beyond the hospital • Oversee and advise on patient safety matters particularly in regards to capacity and pinch points which might affect maximizing patient flow • Liaise with senior clinical colleagues to ensure they have their departmental plans ready to activate and determine whether they require any support	
3	• Pre-alert if appropriate Llandudno Hospital, agreed with the Emergency Consultant	

	• Identify a Senior Registrar (incident dependent) i.e. surgical / medical to be placed in charge of patients admitted from the incident to the dedicated ward • Write a summary report for the Chief Executive	

YG MAJOR INCIDENT ACTION CARD:
Nursing Incident Commander (NIC)

Role:

The Nursing Incident Commander (NIC) will initially be the Clinical Site Manager (A) and will have responsibility for working with the HIC and the MIC to support the operational response.

The Nursing Incident Commander (NIC) will be the Site Director of Nursing/Head of Nursing in hours and initially the CSM out of hours. They will have responsibility for co-ordinating the hospital's response to the incident.

If the CSM is the 1st responder to the incident. The CSM will be responsible for contacting the HIC, ED Consultant and the Physician on Call to determine the appropriate hospital response having considered the METHANE Report received from WAST. If the NIC has responded, they will be responsible for contacting the HIC, ED Consultant and Physician on Call.

The Nursing Incident Commander (NIC) can be contacted on **03000 840491.**

	Action	**Completed By: Date & Time**
1	**Major Incident Standby** NIC will support HIC, MIC and ED Consultant in determining the appropriate hospital response having considered the METHANE Report received from WAST. If HMT Cascade responders (appendix or action card) may also be available if appropriate to: • Establish Hospital Command Centre • Ensure that up to date bed information on bed states across all wards is available. • Designate a Nurse to take over the CSM role (patient flow) until relieved by a senior member of the CSM Team/Head of Nursing, Medicine. • Develop a plan to optimise capacity to admit and discharge patients taking account of patient flow in site specific diagrams and staffing demand. • Ensure that processes are in place to bring in additional nursing staff and equipment.	

2	**Major Incident Declared** • Activate arrangements outlined at Major Incident Standby. • Oversee patient flow, discharge outpatients, ED, Surgery, Medicine and Women and Children's Services. • Maintain up to date information on bed status. • Consider redeployment of staff to critical areas. • Liaise with the Consultant Anaesthetist on-call (who will attend the ED) and the Hospital Command Centre to identify availability of ITU/ICU beds in neighbouring hospitals. • Oversee and support the creation of capacity in ED by expediting discharge. • Issue radios/mobile phones from CSM Office to key staff, as a minimum: • ED Consultant • ED Senior Nurse on site • Senior Porter • CSM's • Bronze Managers x 5	

This card is for the Hospital Command Centre (HCC)

YG MAJOR INCIDENT ACTION CARD:
Hospital Command Centre (HCC)

Role:

The HCC is a designated area which will be established by the CSM / Bronze On-call Manager to facilitate co-ordination of the whole hospital response. The HCC will be managed by the Hospital Command Centre consisting of:

Ysbyty Gwynedd Board Room (03000 850823)

- Hospital Incident Commander (HIC) - 03000 840490
- Medical Incident Commander (MIC) - 03000 840492
- Nursing Incident Commander (NIC) - 03000 840491
- Duty Administrator (Bronze On-call) - 03000 840493
- 4 x Loggists

Ysbyty Gwynedd Executive Committee Room

- 4 x members of the On-Call Management Team
- 2 x Loggists
- 6 x Runners
- Police Liaison Officer Desk
- Ambulance Liaison Officer Desk

Major Incident Hierarchy

To ensure effective response which is supportive of the overall multi-agency response BCU adopts a Bronze (Operational); Silver (Tactical) and Gold (Strategic) Model.

Essentially this means that Bronze will manage the operational response at a hospital or community level, Silver will co-ordinate multiple hospital responses and deliver a primary care response. Gold will set strategy and ensure a multi-agency approach.

	Action	**Completed By: Date & Time**
1	**Definition of a Hospital Major Incident** The number or types of casualties require special arrangements to be implemented by the hospital and jeopardise the delivery of core hospital services.	

<table>
<tr><td>2</td><td>Initial Activation (Major Incident Standby)

Upon receipt of notification of an incident, the HIC, NIC and MIC, together with the ED Consultant will determine an appropriate level of hospital response, following review of relevant action card.</td><td></td></tr>
<tr><td>3</td><td>Activation (Major Incident Declared)

Upon receipt of notification of a Major Incident, the CSM (NIC) will set up the HCC, the HIC and MIC will attend the HCC and collect and fulfil their action cards.</td><td></td></tr>
<tr><td>4</td><td>Formal Handover Process

To ensure an appropriately skilled and experienced Hospital Management Team, the initial responders can formally handover to more senior colleagues using a formal handover process.

<table>
<tr><td rowspan="4">Switchboard notifies</td><td>Initial Response</td><td rowspan="4">Handover procedure</td><td>On-going Response</td><td>Role</td></tr>
<tr><td>CSM</td><td>ADNS (nursing)
Site Lead Nurse</td><td>NIC (can assume the role of HIC if deemed more appropriate)</td></tr>
<tr><td>Bronze On-call</td><td>Hospital Director/ Managing Director/Directorate Manager with acute operational experience.</td><td>HIC</td></tr>
<tr><td>Physician On-call</td><td>AMD</td><td>MIC</td></tr>
</table>
</td><td></td></tr>
<tr><td></td><td></td><td></td></tr>
</table>

YG MAJOR INCIDENT ACTION CARD:
Duty Administrator Loggist

Role:

NB this role may need to be performed by more than one person – the Hospital Incident Commander (Bronze) will ensure an appropriate allocation of responsibilities depending on the skills of the individuals who are available.

	Action	**Completed By: Date & Time**
1	Report to the Hospital Command Centre and locate the Duty Administrator	
2	Take control of the incident log that he/she has commenced. Time, date and both sign that the log has been handed over	
3	Focus only on this role	
4	Record ALL conversations had, information received/requested and decisions made with rationale by the Duty Administrator	
5	Keep detail specific and as brief as possible	
6	Initial and time each entry	
7	Should the log change hands, repeat the handover procedure in point 3	
8	When incident is over and log actioned to be closed, rule off the log with a **Z** sign off with full signature with date and time	
9	Collate any additional documents, trigger notes, photocopies or other material used by the Duty Administrator	
10	Entries should be clear, intelligible and accurate	
11	All entries to be made in permanent black ink in strict chronological order – you are logging actions and decisions not taking minutes	
12	Begin each entry on a new line but ensure there are NO gaps between lines/entries	
13	Note any non-verbal communication i.e. DA nodded head	
14	DO NOT erase any text – put a single line through, time, date and signed by both parties	
15	DO NOT overwrite any text or use the margins	
16	DO NOT remove any pages from the log(additional sheets to be numbered sequentially and both sides of page used)	
17	Note other persons present at meetings/discussions etc	
18	DO NOT use arrows/dashes etc	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Hospital Incident Commander (HIC) Loggist

Role:

NB this role may need to be performed by more than one person – the Hospital Incident Commander (Bronze) will ensure an appropriate allocation of responsibilities depending on the skills of the individuals who are available.

	Action	**Completed By: Date & Time**
1	Report to the Hospital Commend Centre and locate the Duty Administrator	
2	Take control of the incident log that he/she has commenced. Time, date and both sign that the log has been handed over	
3	Focus only on this role	
4	Record ALL conversations had, information received/requested and decisions made with rationale by the Duty Administrator	
5	Keep detail specific and as brief as possible	
6	Initial and time each entry	
7	Should the log change hands, repeat the handover procedure in point 3	
8	When incident is over and log actioned to be closed, rule off the log with a **Z** sign off with full signature with date and time	
9	Collate any additional documents, trigger notes, photocopies or other material used by the Duty Administrator	
10	Entries should be clear, intelligible and accurate	
11	All entries to be made in permanent black ink in strict chronological order – you are logging actions and decisions not taking minutes	
12	Begin each entry on a new line but ensure there are NO gaps between lines/entries	
13	Note any non verbal communication i.e. DA nodded head	
14	DO NOT erase any text – put a single line through, time, date and signed by both parties	
15	DO NOT overwrite any text or use the margins	
16	DO NOT remove any pages from the log(additional sheets to be numbered sequentially and both sides of page used)	
17	Note other persons present at meetings/discussions etc	
18	DO NOT use arrows/dashes etc	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Medical Incident Commander (MIC) Loggist

Role:

NB this role may need to be performed by more than one person – the Hospital Incident Commander (Bronze) will ensure an appropriate allocation of responsibilities depending on the skills of the individuals who are available.

	Action	**Completed By: Date & Time**
1	Report to the Hospital Commend Centre and locate the Duty Administrator	
2	Take control of the incident log that he/she has commenced. Time, date and both sign that the log has been handed over	
3	Focus only on this role	
4	Record ALL conversations had, information received/requested and decisions made with rationale by the Duty Administrator	
5	Keep detail specific and as brief as possible	
6	Initial and time each entry	
7	Should the log change hands, repeat the handover procedure in point 3	
8	When incident is over and log actioned to be closed, rule off the log with a **Z** sign off with full signature with date and time	
9	Collate any additional documents, trigger notes, photocopies or other material used by the Duty Administrator	
10	Entries should be clear, intelligible and accurate	
11	All entries to be made in permanent black ink in strict chronological order – you are logging actions and decisions not taking minutes	
12	Begin each entry on a new line but ensure there are NO gaps between lines/entries	
13	Note any non-verbal communication i.e. DA nodded head	
14	DO NOT erase any text – put a single line through, time, date and signed by both parties	
15	DO NOT overwrite any text or use the margins	
16	DO NOT remove any pages from the log (additional sheets to be numbered sequentially and both sides of page use)	
17	Note other persons present at meetings/discussions etc	
18	DO NOT use arrows/dashes etc	

BLANK SHEET

This card is for the Nursing Incident Commander (NIC) Loggist

YG MAJOR INCIDENT ACTION CARD:
Nursing Incident Commander (NIC) Loggist

Role:

NB this role may need to be performed by more than one person – the Hospital Incident Commander (Bronze) will ensure an appropriate allocation of responsibilities depending on the skills of the individuals who are available.

	Action	**Completed By: Date & Time**
1	Report to the Hospital Commend Centre and locate the Duty Administrator	
2	Take control of the incident log that he/she has commenced. Time, date and both sign that the log has been handed over	
3	Focus only on this role	
4	Record ALL conversations had, information received/requested and decisions made with rationale by the Duty Administrator	
5	Keep detail specific and as brief as possible	
6	Initial and time each entry	
7	Should the log change hands, repeat the handover procedure in point 3	
8	When incident is over and log actioned to be closed, rule off the log with a **Z** sign off with full signature with date and time	
9	Collate any additional documents, trigger notes, photocopies or other material used by the Duty Administrator	
10	Entries should be clear, intelligible and accurate	
11	All entries to be made in permanent black ink in strict chronological order – you are logging actions and decisions not taking minutes	
12	Begin each entry on a new line but ensure there are NO gaps between lines/entries	
13	Note any non-verbal communication i.e. DA nodded head	
14	DO NOT erase any text – put a single line through, time, date and signed by both parties	
15	DO NOT overwrite any text or use the margins	
16	DO NOT remove any pages from the log(additional sheets to be numbered sequentially and both sides of page used)	
18	Note other persons present at meetings/discussions etc	
19	DO NOT use arrows/dashes etc	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Emergency Department (ED) Consultant

Role:

The Lead ED Consultant/Senior Doctor on-call will be contacted by the Clinical Site Manager who, together with the Bronze on-call, will determine the appropriate hospital response having considered the METHANE Report received from WAST.

It is important to recognise that the key responsibilities of the Lead ED Consultant on-call lie in the co-ordination, tasking and supervision of medical staff and clinical services and *not* in the direct provision of hands-on care.

	Action	**Completed By: Date & Time**
1	**Major Incident Standby** The Lead Consultant will: • Conduct a quick Board Round. • Attend the Hospital Command Centre to determine appropriate hospital response. • Ensure that the ED has been informed of the standby alert. • Inform the Hospital Command Centre of his/her attendance within the ED and continue to exchange current information on the potential incident. • Liaise with Consultant in Anaesthetics (+/- other relevant specialities as determined by nature of incident, e.g. paediatrics) and provide current information on the potential incident.	
2	**Major Incident Declared** The Lead Consultant will: • Attend the Hospital Command Centre to determine appropriate hospital response – if not already done at standby. • Inform the Hospital Command Centre of his/her attendance within the Emergency Department and continue to exchange current information on the incident.	

	• Ensure that a member of clerical or support staff has commenced calling other ED medical staff by telephone as decided by the Lead ED Consultant. • Liaise with the Hospital Command Centre if additional nursing or medical staff need to be requested from other departments or wards. • Ensure there is a designated Casualty Triage Officer – e.g. experienced Emergency Physician or other doctor with appropriate training. • Delegate overseeing of the treatment of Priority 1 patients within the ED to appropriate Senior Doctor. • Ensure that suitable staff (e.g. ED doctor/nurse or appropriate speciality e.g. medicine) are allocated to ED patients not involved in the major incident and that their transfers/discharge are being expedited. Low-acuity patients should be redirected if possible to their GP or an alternative MIU/ED. • Advise the Hospital Command Centre if it looks likely that the ED is at risk of being significantly overwhelmed and the possible need to divert patients to other Emergency Departments. • Maintain close contact/liaison with the Hospital Command Centre and the Senior Nurse in the ED, especially in the Resus area. • Following stand-down supervise the return to normal function • Ensure a debrief takes place within a suitable timeframe (not usually the same day).	

YG MAJOR INCIDENT ACTION CARD:
Emergency Department (ED) Nurse in Charge

Role:

The Nurse in Charge of the Emergency Department will support the ED Consultant in the preparation of the department and liaise with the Senior CSM in relation to patient flow and ensuring clinical staff resources are sufficient to facilitate patient flow.

	Action	**Completed By: Date & Time**
1	**Action** The Nurse in Charge of the Emergency Department will: • Put on NIC tabard • Obtain the Departmental Action Cards and Distribute • The NIC will create a new MAJAX on Symphony via 'Declare Incident' – please see Symphony Guide 'User Guide 2020 – Majax' • The NIC will work alongside the ED Consultant in Charge • The NIC will not get involved in direct patient care, the role is to co-ordinate activity	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Emergency Department (ED) Receptionist

Role:

On receiving notification of **'Major Incident Standby'** you will:

Call 4 members of staff from the staff list (available in the Major Incident Folder) ensuring that we do not call staff working the next two shifts as the incident may last for a long period of time.

	Action	**Completed By: Date & Time**
1	On Receiving notification of **'Major Incident Declared'**, ensure that the two Receptionists set up the ED Reception area as follows:	
2	**First Receptionist:** • First Receptionist to remain at the reception desk to register 'normal' patients and any casualties self-presenting from the incident • Any self-presenting casualties from the incident must be registered under the incident and the Triage Nurse on duty informed as soon as possible, location to be recorded as 'waiting room' • All patients who have been involved in this incident, even the 'walking wounded' must be booked onto Symphony under this incident, to allow for traceability and police casualty enquiries.	
3	**Second Receptionist** • Second Receptionist to relocate to the desk and computer under the ambulance bay window • Log into Symphony and admit patients under the Incident (which will have been set up by NIC). Symphony 'User Guide 2020 – Majax' gives step by step instructions and is available in the Major Incident folder As casualties arrive and you receive details: • Enter details provided onto Symphony under the incident • Enter the casualty's location as indicated on the 'reception sheet' • Ensure that the major incident tab is completed for each casualty	
4	• For any 'Unknown' patients, please follow the 'Registering Unknown Patients on WPAS/Symphony in ED Reception' Guide, which is located on the wall to the right of the front reception desk.	

5	• When the additional staff arrive, they may be required to work on the main reception desk, or deployed as admin support for nursing and medical staff during the incident.	

YG MAJOR INCIDENT ACTION CARD:
Estates and Facilities Cell

Role:

The role of the Estates and Facilities Cell is to ensure the efficient management and provision of the services listed below. This role will be carried out by a member of the Bronze On-call (BOC) Team.

Consider an appropriate BOC Manager with a knowledge of Estates / Facilities.

	Action	Completed By: Date & Time
1	**Major Incident Declared** • Proceed to the Hospital Command Centre (HCC) to be briefed and tasked by the Hospital Incident Commander (HIC)	
2	• The BOC taking on the role of co-ordinating Estates and Facilities will then co-ordinate and oversee matters relating to: • Hotel Services including Portering, Domestic Services, Stores and Linen • Catering • Continuity of utility services • Security if necessary • Provision and distribution of equipment as required	
3	• If Appropriate, hand over to Senior Site Estates and Facilities Officers	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Information Management Cell

Role:

The role of the Information Management Cell is to ensure efficient management of all information relating to the incident and will be carried out by a member of the Bronze On Call (BOC) Team.

	Action	Completed By: Date & Time
1	**Major Incident Declared** • Proceed to the Hospital Command Centre to be briefed and tasked by the Hospital Incident Commander, if not already done so. • Contact Loggists and request attendance at the HCC and other key areas. • The BOC taking on the role of Information Manager will then co-ordinate and oversee matters relating to press and media enquiries; in close liaison with the Hospital Director and Corporate Communications. • Designate responsible officer to ensure tracking of major incident casualties. • Co-ordinate and oversee the communication between enquiries and or the relatives area, with appropriate services. • Provide a point of contact for Police Liaison. • Provide a point of contact for Switchboard and any support required by switchboard. • Write situation reports on behalf of the Hospital Management Team.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Non-Acute Services Cell

Role:

The role of the Non Acute Services Cell is to ensure the efficient management and provision of non-acute services as listed below. This role will be carried out by a member of the Bronze on Call (BOC) Team.

	Action	Completed By: Date & Time
1	**Major Incident Declared** • Proceed to the Hospital Command Centre (HCC) to be briefed and tasked by the Hospital Incident Commander (HIC) • The BOC taking on the role of Non Acute Services Manager will co-ordinate and liaise with the following: • IHC Director (West) and IHC Matron Community and District Nursing of the day who will co-ordinate with Community Nursing Teams who will then liaise with social services, voluntary services and the third sector • Primary Care Support to ensure co-ordination of primary care • Mental Health Division to ensure co-ordination of Mental Health Services and to support ED – admission wards to co-ordinate patients awaiting assessment. Patients to be taken to the Hergest Unit. • Womens Services make contact with HCC to provide support if required	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Staffing Cell

Role:

The role of the Staffing Cell is to ensure adequate staffing to maintain a safe working environment for the immediate period and the longer term. This role will be carried out by a member of the Bronze On Call (BOC) Team.

	Action	Completed By: Date & Time
1	**Major Incident Declared** • Proceed to the Hospital Command Centre (HCC) to be briefed and tasked by the Hospital Incident Commander (HIC) • The BOC taking on the role of Staffing Support Manager will then co-ordinate and oversee matters relating to: • The provision of medical staffing as directed by the IHC Management Team. o Ensure that adequate nursing staff are available throughout the incident in liaison with Nurse Bank o Human Resources – ensure sufficient welfare rest periods for staff involved in the incident o Provision of Occupational Health Support o Welfare of staff and provision of suitable hot debriefs o Infection Prevention and Control o Ensure Mental Health Team are requested to support staff affected by the incident	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Support Services Cell

Role:

The role of the Support Services Cell is to ensure efficient management and provision of the services listed below. This role will be carried out by a member of the Bronze On Call (BOC) Team.

	Action	Completed By: Date & Time
1	**Major Incident Declared** • Proceed to the Hospital Command Centre (HCC) to be briefed and tasked by the Hospital Incident Commander (HIC) • The BOC taking on the role of Support Services Manager will then co-ordinate and oversee matters relating to: o Co-ordination with the Police Documentation Team o The Relatives Centre o Medical Records o HSDU/CSSD o Pharmacy requirements o Physiotherapy o Occupational Therapy o IM&T issues o Radiology o Pathology o Mortuary o VIP arrangements	

BLANK SHEET

This card is for the Communications Officer

YG MAJOR INCIDENT ACTION CARD:
Media Liaison Office

Role:

When a Major Incident is declared, the Incident Manager (Bronze On-call) will be responsible for identifying a member of staff to manage the media response. In hours, this would be a member of the Corporate Communications Team.

Out of hours the nominated member of staff will manage media liaison until a member of the Corporate Communications Team can take over.

	Action	**Completed By: Date & Time**
1	A Media Liaison Office will be set up on the Executive Corridor, Ysbyty Gwynedd. The contact details are: **Telephone: 03000 840008** **E-mail: bcuhbpressdesk@wales.nhs.uk**	
2	Follow the guidance in the Major Incident Communications Plan.	
3	Once the Media Liaison Office has been established, the Corporate Communications Team will: • Get a full situation report from the Hospital Incident Commander (HIC) • Liaise with the Assistant Director of Communications and Engagement and agree at what level the incident will be managed (Site, Health Board or LRF) • Agree any communications for staff/media as required with the Hospital Incident Commander (HIC) or assist the Assistant Director of Communications and Engagement to do so • Agree any liaison with media on site, including identifying a suitable area for media briefings if required • Keep a log of all: ○ Media enquiries ○ Messages/statements issued internally and externally and the channels used	

BLANK SHEET

National Ambulance Resilience Unit
NARU

NHS

JESIP

MAJOR INCIDENT INITIAL ACTION CARDS



June 2021 - OFFICIAL
Pocket Version 2.0

National Ambulance Resilience Unit
NARU

ASSOCIATION OF AMBULANCE CHIEF EXECUTIVES

This document is the intellectual copyright of the National Ambulance Resilience Unit (NARU).

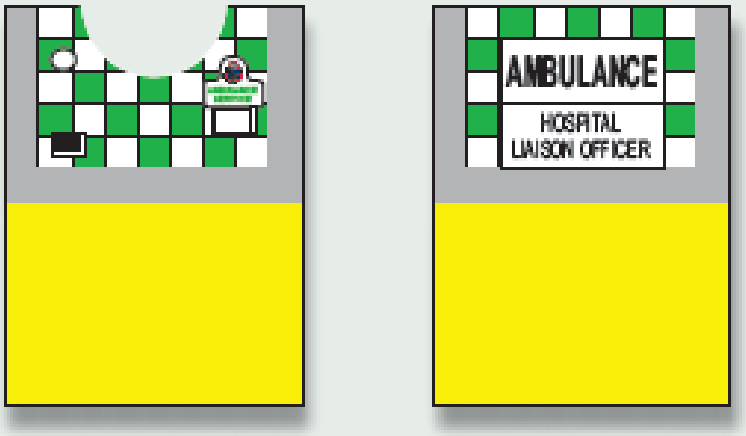
BLANK SHEET

National Ambulance Resilience Unit
NARU

OFFICIAL

NHS

14.0 Hospital Ambulance Liaison Officer



ACTION	DESCRIPTION	COMPLETED	TIME
1	Report to the designated receiving hospital under the direction of the coordinating Ambulance Control.		
2	Check communications / Talk Group and start a timed log. If cross border ensure the use of mutual aid Talk Group.		
3	Don high-visibility tabard inscribed Hospital Liaison Officer. Switch to dedicated Talk Group.		
4	Establish and maintain communications between the receiving hospital and the Ambulance Control.		
5	Liaise with the Hospital Emergency Department Officer and the Police Casualty Bureau Officer.		

CONTINUED OVERLEAF



National Ambulance Resilience Unit
NARU

OFFICIAL

NHS

HOSPITAL AMBULANCE LIAISON OFFICER

14

ACTION	DESCRIPTION	COMPLETED	TIME
6	Liaise with the MERIT (where available) prior to deployment.		
7	Control and direct Ambulance activities to manage the prompt turnaround from the hospital back to the incident scene if required.		
8	Ensure release of Ambulance equipment by the hospital and arrange its return to an appropriate location as required. Ensure the welfare requirements of staff are met prior to return to scene.		
9	Co-ordinate and log the return of all Ambulance equipment not available for release during the incident.		
10	Maintain a log of casualties arriving at the hospital department - HALO Log.		
11	Arrange for the availability of consumable Ambulance equipment for Ambulance crews to replenish their vehicles at the hospital.		
12	Remain at the hospital subsequent to **"Stand Down"** to maintain continuing liaison in managing the continuing demands on the hospital and Ambulance resources for discharges and secondary transfers.		
13	Be prepared to attend a debrief after the incident.		

National Ambulance Resilience Unit
NARU

OFFICIAL

NHS

Hospital Ambulance Liaison Log

HOSPITAL AMBULANCE LIAISON LOG

TO BE COMPLETED AT A RECEIVING HOSPITAL

VEHICLE CALL SIGN	ARRIVAL TIME	CASUALTY NAME OR NUMBER	TRIAGE NUMBER	ADULT M / F	CHILD M / F	CLEAR TIME	REDEPLOYED SCENE	OTHER

CONTINUED OVERLEAF

HOSPITAL AMBULANCE LIAISON OFFICER

14

National Ambulance Resilience Unit
NARU

OFFICIAL

NHS

14

HOSPITAL AMBULANCE LIAISON OFFICER

HOSPITAL AMBULANCE LIAISON LOG

TO BE COMPLETED AT A RECEIVING HOSPITAL

VEHICLE CALLSIGN	ARRIVAL TIME	CASUALTY NAME OR NUMBER	TRIAGE NUMBER	ADULT M / F	CHILD M / F	CLEAR TIME	REDEPLOYED SCENE / OTHER

This card is for the Police Liaison Officer

YG MAJOR INCIDENT ACTION CARD:
Police Documentation Team

Role:

A Police Documentation Team should be deployed to each reception, rest centre and hospital receiving survivors/evacuees from an emergency or major incident.

The purpose of this team is to document casualty and survivors/evacuees' details, which will be fed into the Casualty Bureau process.

	Action	Completed By: Date & Time
1	• A Police Documentation Team should be deployed to the hospital receiving survivors / evacuees from an emergency or major incident. The purpose of this team is to document casualty and survivors / evacuees details, which will be later fed into the Casualty Bureau process. Officers will be deployed to form Documentation Teams, under the command of the 'Bronze Survivor Reception/Rest Centre' or 'Hospital Bronze', both of which are referred to as 'Documentation Team Leader'. They will report to the 'Tactical Commander' until or unless a 'Senior Identification Manager' (SIM) is appointed, when they will report to the SIM as part of the overall identification and investigation process. The team will be based in the Beaumaris Room, Education Centre , Ysbyty Gwynedd. • Major Incident boxes, which are red boxes with red lids, will be labelled with "MAJOR INCIDENT HOSPITAL DOCUMENTATION". A Major Incident Box will be located at each of the three main North Wales Hospital sites and contain necessary instructions and paperwork. They have sufficient instructions, paperwork and equipment to enable direct officers to set up either a hospital or Survivor Reception Documentation Team. • The Major Incident Box for Ysbyty Gwynedd will be located within the Major Incident Cupboard in the Boardroom. • The Documentation Team should consist of four officers:	

Team Leader (Operational)

- Responsible to the Tactical Commander until or unless a Senior Identification Manager (SIM) is appointed, when they will report to the SIM as part of the overall identification process.
- Assume responsibility for the documentation of casualties taken to a particular 'Designated Receiving Hospital'.
- Assume responsibility for the documentation of persons being reported missing or unaccounted for following the incident.
- Assume responsibility for the documentation of other information such as uninjured survivors along with friends and family.
- Liaise with Hospital Command Centre and hospital staff on matters relating to documentation and identification of casualties / survivors / missing from home / friends and family.
- Maintain communication link with the Tactical Commander, Force Control Room, Casualty Bureau (if operating), and if necessary, the Rest / Survivor Reception Centre.
- Ensure that completed casualty, missing from home, survivor and message forms are forwarded to the Casualty Bureau via Liaison Officer.
- Allocate the following duties: Documentation Officers, Liaison Officer and Property Officer.
- If required, arrange for additional security at the hospital.
- Initially assume responsibility for persons who are dead on arrival or die at the hospital and liaise with Mortuary Team.
- Record decisions and instructions along with their rationale in the Major Incident Log Books (MIR 22).
- Issue Major Incident Log Books (MIR 22) to team members.

Documentation Officers x 2

- Responsible to the Documentation Team Leader.
- Work alongside Hospital Documentation staff and complete the casualty / survivor / evacuee record (blue form) in respect of all casualties and uninjured survivors who haven't already been documented at a Survivor Reception Centre.
- Hand completed forms to the Liaison Officer.
- Utilise the Major Incident Public Portal (MIPP) app to document any relevant information for the Casualty Bureau: https://mipp.police.uk
- Update Liaison Officer with any amended information for onward transmission to the Casualty Bureau.
- Perform any other functions as directed by the Documentation Team Leader.

Liaison Officer x 1 (to maintain communication)

- Responsible to the Documentation Team Leader.
- Maintain communication links with Ops Planning Department, Tactical Commander, Force Control Room, Casualty Bureau (if

	operating) and if necessary, Reception Centre Documentation Team. • Receive completed casualty / survivor / evacuee record forms. • Forward completed forms to the Casualty Bureau as soon as practicable. Consider using email or employing the use of a dedicated driver e.g. Special Constable, PCSO, (avoid depleting members of the Bronze Hospital Team). • Perform any other functions as directed by the Team Leader. **Property Officer x 1 (Hospital Team only)** • Responsible to the Documentation Team Leader (Bronze). • Record and store property brought in with casualties if: o they are unidentified and die o the property is required for forensic examination o they are suspected of involvement in crime o the property does not belong to the injured • Record property using Major Incident Property Registers, ensuring that all property is suitably bagged, exhibit labelled and sealed. • Perform other functions as directed by the Documentation Team Leader. More Police Officers may be required depending on the scale of the major incident and police investigation.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Discharge Support Team

Role:

Major Incident Standby

During Normal Working Hours

The Discharge Support Team will liaise with wards and Junior Medical Staff to identify patients who could be safely discharged home or to another hospital.

Provide information to the Clinical Site Manager in the Emergency Department (ED) and Hospital Command Centre (HCC) about the discharge and transfer of patients and the resulting bed availability.

Out of Normal Working Hours

The Discharge Support Team will be contacted by the Acute Matron Flow / Senior Clinical Site Manager or CSM.

The Discharge Support Team will liaise with Wards and Junior Medical Staff to identify patients who could be safely discharged home, other discharge destination or to another hospital.

Provide information to the Clinical Site Manager and Hospital Command Centre about the discharge of patients and the resulting bed availability.

	Action	**Completed By: Date & Time**
1	**Major Incident Declared** **During Normal Working Hours** The Discharge Support Team will liaise with wards and Junior Medical Staff to identify patients who could be safely discharged home, other discharge destination or to another hospital.	
2	Liaise with the Nursing Staff regarding patients who are to be discharged or transferred to other hospitals. Provide information to the CSM and HCC about the discharge of patients and the resulting bed availability.	

3	**Out of Normal Working Hours** The Discharge Support Team will be contacted by the Senior Clinical Site Manager or CSM (B). The Discharge Support Team will liaise with wards and Junior Medical Staff to identify patients who could be safely discharged home or to another hospital. Liaise with the nursing staff regarding patients who are to be discharged or transferred to other hospitals. Provide information to the Clinical Site Manager and Hospital Command Centre about the discharge of patients and the resulting bed availability. At the direction of the Hospital Command Centre, the Clinical Site Manager and the Discharge Support Team will liaise with wards, junior medical staff and Portering staff to affect the transfer or discharge of patients. Wards should be advised that patients being discharged should be taken to the Transfer Lounge where they will be cared for until transport home can be arranged.	

YG MAJOR INCIDENT ACTION CARD:
Pharmacy – In Hours

Role:

On receipt of standby alert, the Pharmacist will report to the Hospital Command Centre (HCC).

	Action	**Completed By: Date & Time**
1	Alert senior member of the Pharmacy Senior Management Team (including duty manager) of the major incident.	
2	A member of the Pharmacy Senior Management Team to report to Hospital Command Centre to receive further information and collect pharmacy tabards.	
3	Establish a pharmacy base in Consulting Room 10 in main outpatients. Three pharmacy staff will be based in Consulting Room 10 an allocated to P1, P2 and P3. Staff to wear corresponding tabards. Other pharmacy staff will be based in the main Pharmacy.	
4	Maintain a log of actions, phone calls and supplies provided.	
5	Maintain a log of actions, phone calls and supplies provided.	
6	Frequently assess staffing and call any off duty staff members as necessary.	
7	Pharmacy staff to support discharge of patients if not required to directly support Major Incident.	
8	Review progress frequently and ensure staff have appropriate breaks depending on the length of shifts etc.	
9	Set up a rota to maintain continuity of service as required.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Pharmacy – Out of Hours

Role:

On receipt of standby alert, the Pharmacist will report to the Hospital Command Centre (HCC).

	Action	**Completed By: Date & Time**
1	Notify the Gold on-call for pharmacy before leaving home. Proceed immediately to the hospital.	
2	Report to Hospital Command Centre once in the hospital to receive further information and collect pharmacy tabards.	
3	Pharmacy Gold on-call to alert Senior member of pharmacy staff (see list on Gold on call rota). Call in at least 3 extra staff as required depending on the nature of the major incident. Off duty members of staff should not attend until requested.	
4	Establish a pharmacy base in Consulting Room 10 in main outpatients. Three pharmacy staff will be based in Consulting Room 10 an allocated to P1, P2 and P3. Staff to wear corresponding tabards. Other pharmacy staff will be based in the main Pharmacy (numbers depending on incident and availability).	
5	Maintain a log of actions, phone calls and supplies provided.	
6	P1, P2 and P3 assigned pharmacy staff will order stock drugs for areas as required depending on incident. Depending on numbers of staff available, pharmacy staff will support distribution of medicines.	
7	Pharmacy staff to support discharge of patients if not required to directly support Major Incident.	
8	Pharmacy staff to support discharge of patients if not required to directly support Major Incident.	
9	Review progress frequently and ensure staff have appropriate breaks depending on the length of shifts etc.	

BLANK SHEET

This card is for the Catering Department

YG MAJOR INCIDENT ACTION CARD:
Catering Department

Role:

There are members of staff on site between 6.00am and 9.00pm. During these times the Catering Department will be informed of Major Incident Standby or Major Incident Declared by the Hospital Command Centre.

Out of hours, Catering Managers can be contacted through Switchboard.

	Action	Completed By: Date & Time
1	**Major Incident Standby** **Out of Normal Working Hours:** • The Department Manager/Deputy should attend the site. • Notify department members of staff that the hospital has been placed on Major Incident standby and that they may be required to attend the site. **During Normal Working Hours:** • Notify department members of staff that the hospital has been placed on Major Incident standby and ask if they would be available to attend the site if required.	
2	**Major Incident Declared** **During normal working hours:** • Call in additional staff as required. • The Department will make preparations to provide beverages, refreshments for patients, staff and relatives using professional judgement and will inform the Hospital Command Centre of the proposed arrangements. • Voluntary establishments should be contacted via Switchboard, to provide additional catering services from their facilities, which should be staffed by the volunteers themselves.	

	• Specific areas will require catering support with deliveries of food and drink – they are likely to be the ED, Outpatients Departments, Hospital Command Centre, Police, Documentation Team, Operating Theatres and the Media area – it is likely that staff within these areas will be working extended shifts and will be unable to attend the on-site catering facilities. • Make available Dining Room floor space in order to provide additional support as required.	

YG MAJOR INCIDENT ACTION CARD:
Facilities Department

Role:

There are members of Facilities staff on site 24hrs.

Out of Hours, there are staff on shift who will be informed of Major Incident Standby or Major Incident Declared by the Hospital Command Centre.

	Action	Completed By: Date & Time
1	**Major Incident Standby** **Out of Normal Working Hours:** • Outside of normal working hours, Facilities Managers can be contacted via Switchboard should their assistance be required by the Bronze on call team. • The Department Manager should attend the site. • Notify department Managers and Supervisors that the hospital has been placed on Major Incident standby and that they may be required to attend the site. **During Normal Working Hours:** • Notify department members of staff, that the hospital has been placed on Major Incident standby and ask if they would be available to attend the site if required.	
2	**Major Incident Declared** A Manager responsible for Facilities will attend the Hospital Command Centre, to be briefed and tasked by the Hospital Incident Commander / Bronze On-call. • The Facilities Manager will then co-ordinate and oversee matters relating to the continuity of Catering and Hotel Services (including Portering Domestic, Stores and Linen services) • Arrange the call out of sufficient Managers, Supervisors and staff as appropriate.	

	• Ensure that regular updates/briefings are provided to the HIC. • Make available Dining Room floor space in order to provide additional support as required.	

YG MAJOR INCIDENT ACTION CARD:
Estates Department

Role:

There are members of Estates staff on site between 08.00 and 16.30.

Out of Hours, there are staff on shift who carry Bleep 312, who will be informed of Major Incident Standby or Major Incident Declared by the Hospital Command Centre.

Out of hours, On-call Estates Officer can be contacted through Switchboard.

	Action	Completed By: Date & Time
1	**Major Incident Standby** **Out of Normal Working Hours:** • Estates have Estates Officers on-call, outside normal working hours and they can be contacted via switchboard, should their assistance be required by the Bronze on call team. Facilities Managers can be contacted through Switchboard. • The Department Manager should attend the site. • Notify department Managers and Supervisors that the hospital has been placed on Major Incident standby and that they may be required to attend the site. **During Normal Working Hours:** • Notify department members of staff that the hospital has been placed on Major Incident standby and ask if they would be available to attend the site if required.	
2	**Major Incident Declared** An Estates Officer / Manager responsible for Estates will attend the Hospital Command Centre, to be briefed and tasked by the Hospital Incident Commander / Bronze On-call. • The Estates Officer / Manager will then co-ordinate and oversee matters relating to continuity of utility services • Arrange the call out of sufficient staffing as appropriate. • Ensure that regular updates/briefings are provided to the HIC.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
CSSD / HSDU Department

Role:

CSSD/HSDU Department will ensure that all equipment requiring sterilisation is collected and processed.

	Action	Completed By: Date & Time
1	**Major Incident Standby** **During normal working hours:** A member of the department should collect all available theatre and Emergency Department items requiring sterilisation. **Out of normal working hours:** The Head of Department/Deputy should be contacted at home by the Hospital Command Centre (HCC) and asked to attend CSSD/HSDU	
2	**Major Incident Declared** **During normal working hours:** • Plans to extend working hours should be implemented. • Call in additional staff as required. • All available theatre and ED equipment will be processed. • Either the CSSD/HSDU Manager or Deputy will liaise with theatres and ED to ensure equipment is processed and stocks replenished as required. **Out of normal working hours:** • Additional staff should be called in as required. • Plans to extend working hours should be implemented. • All available theatre and ED equipment will be processed. • Either the HSDU Manager or Deputy will liaise with theatres and ED to ensure equipment is processed and stocks replenished as required.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Junior Doctor / Registrar

Role:

All Doctors carrying an on-call bleep for crash calls will receive notification that the hospital is on major incident standby in or out of hours. The agreed term is:

"The Hospital has been placed on Major Incident Standby and is responding to a Major Incident. Please activate your full Major Incident Response."

	Action	Completed By: Date & Time
1	**During Working Hours** • Once notification is received: • Cascade this information to colleagues working within the same Directorate who are not holding a crash bleep to ensure full awareness of the situation.	
2	**Actions** • Inform the Hospital Command Centre of your awareness of the declared Major Incident Standby. • Continue to perform normal services to existing patients on the wards unless directed by the Consultant Physician in the Hospital Command Centre to identify current patients who could be safely transferred or discharged in order to ensure bed availability within the hospital.	
3	**Do Not** • Attend the Emergency Department unless your action card requires you to do so. • Attend Switchboard. • Attend the Hospital Command Centre.	
4	**Out of Hours** • Extra medical staff will be called in, dependant on departmental action cards. Do not inform colleagues out of hours.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Consultant Orthopaedic Surgeon On-Call

Role:

It is important to recognise that the key responsibilities of the Consultant on-call lie in the management and co-ordination of medical staff and clinical services and not in the direct provision of hands-on care.

	Action	Completed By: Date & Time
1	**Major Incident Standby** The Consultant on-call will: • Attend the Orthopaedic Outpatients. • Inform the Hospital Command Centre of his/her attendance at the Orthopaedic Outpatients and obtain current information on the potential incident. • Forewarn the Department of the possible Major Incident declaration.	
2	**Major Incident Declared** The Consultant on-call will: • Attend the See and Treat Area – Emergency Department • Inform the Hospital Command Centre of his/her attendance at the Orthopaedic Outpatients and obtain current information on the potential incident. • Alert the Orthopaedic Department to the Major Incident declaration. • Ensure the continuous observation and secondary triage of patients whilst in the department. If the patient's condition deteriorates to Priority 1 the Hospital Command Centre must be informed of the need to transfer the patient. The Hospital Command Centre will authorise transfer back to Priority 1 area as soon as capacity in that area allows. • Oversee and provide assistance in the treatment of Priority 2 patients within the Department. • Liaise with Senior Clinical Site Manager/Acute Matron, Flow with regard to the call out of additional medical staff.	

	• Liaise with the Consultant Anaesthetist and, when the incident involves children, the Consultant Paediatrician on call (both of whom will be located in the Emergency Department), regarding the allocation of orthopaedic medical staff to theatres and the scheduling of patients to theatre. • Liaise with the Hospital Command Centre to obtain updated information on the state of the incident.	

YG MAJOR INCIDENT ACTION CARD:
Consultant Surgeon On-Call

Role:

It is important to recognise that the key responsibilities of the Consultant on-call lie in the management and co-ordination of medical staff and clinical services and not in the direct provision of hands-on care.

	Action	Completed By: Date & Time
1	**Major Incident Standby** The Consultant on-call will: • Attend the Emergency Department present to the ED Lead for briefing and allocation to appropriate areas as needed. • Inform the Hospital Command Centre of his/her attendance at the Emergency Department and obtain current information on the potential incident. • Forewarn the Surgical Department of the possible Major Incident declaration.	
2	**Major Incident Declared** The Consultant on-call will: • Attend the Emergency Department. • Inform the Hospital Command Centre of his/her attendance at the Emergency Department and obtain current information on the incident. • Alert the Surgical Department to the Major Incident declaration. • Ensure the continuous observation and secondary triage of patients whilst in the department. If the patients' condition deteriorates to Priority 1 the Hospital Command Centre must be informed of the need to transfer the patient. • Oversee and provide assistance in the treatment of Priority 2 patients within the Department. • Liaise with Senior Clinical Site Manager/Acute Matron, Flow with regard to the call out of additional medical staff.	

	• Liaise with the Consultant Anaesthetist and, when the incident involves children, the Consultant Paediatrician on call regarding the allocation of surgical medical staff to theatres and the scheduling of patients to theatre. • Liaise with the Hospital Command Centre to obtain updated information on the state of the incident.	

YG MAJOR INCIDENT ACTION CARD:
Consultant Paediatrician On-Call

Role:

The key responsibilities of the Consultant Paediatrician On-call lie in the management and co-ordination of the paediatric response and not in the direct provision of hands on care.

	Action	Completed By: Date & Time
1	**Major Incident Standby** The Consultant on-call will: • Be notified by Switchboard that the hospital is on major incident standby in or out of hours. • Proceed to the Emergency Department if the incident involves children/young people and present to the Seminar Room for briefing by ED Lead and allocation to appropriate areas as needed. • Inform the Hospital Command Centre of his/her attendance within the Emergency Department (if appropriate). • Liaise with the Consultant in the Emergency Department to obtain current information on the incident. • Contact the Senior Nurse Paediatrics on the Children's Ward and assess the state of preparedness to receive children (bed availability / staff / equipment)	
2	**Major Incident Declared** The Consultant on-call will: • Be notified by Switchboard that the hospital is on major incident declared in or out of hours. • Proceed to the Emergency Department if the incident involves children/young people and if not reported during standby. • Inform the Hospital Command Centre of his/her attendance within the Emergency Department (if appropriate).	

	• Liaise with the Consultant in the Emergency Department to obtain current information on the incident and as appropriate with the Consultant Orthopaedic Surgeon and Consultant General Surgeon to co-ordinate the response for incoming children.	

YG MAJOR INCIDENT ACTION CARD:
Blood Science and Transfusion

Role:

The Blood Science and Transfusion Service will ensure that there is sufficient blood products to manage the incident.

	Action	**Completed By: Date & Time**
1	During normal working hours Monday to Friday: 09.00-17.00 hours. On receiving notification of a Major Incident, the Transfusion Service Manager or other senior member of staff initiates the **local** (i.e. West, Central or East) Major Incident Protocol/Procedure (or "Action Card").	
2	Outside of normal working hours, the shift BMS on duty for Haematology/Blood Transfusion and/or Clinical Chemistry, initiates the **local** (i.e. West, Central or East) Major Incident Protocol/Procedure (or "Action Card").	
3.	A suitable representative from Blood Sciences and/or Transfusion will attend the HCC (where possible) to act as the key communication link between services.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Mortuary Manager

Role:

The Mortuary will be notified of the incident by Switchboard. Out of hours the Cellular Pathology Services Manager will need to be contacted via Switchboard.

Please note: All bodies from a major incident are the property of the Coroner.

	Action	Completed By: Date & Time
1	**Major Incident Standby** • Review fridge availability in anticipation of Incident being declared. If necessary, arrange for deceased already in the fridges awaiting collection to be released to Undertakers as soon as possible. • Contact the other 2 sites and ascertain their available fridge space in case it is required.	
2	**Major Incident Declared** • Liaise with the Coroner's Office representative(s) on the management of fatalities from the incident. • Establish the approximate numbers of fatalities and call in other Mortuary and managerial staff if required. • Contact the BCUs Human Tissue Act (HTA) 'Designated Individual' and update them of the situation. This is essential, especially if temporary mortuary facilities may be required. The Designated Individual will contact and liaise with the HTA, Cell Path Manager and the DGM for North Wales Clinical Services. • Mortuary staff to don appropriate PPE and ensure all persons in the area are made aware of potential hazards and have access to essential PPE • As each body arrives ensure they are labelled with the unique major incident number (if received via the Emergency Department) and any personal details that may be available. Do not undress or otherwise interfere with the body.	

	• Affix label with unique major incident number to body bag and record number in records. • Book the deceased in to the day book and place the body in fridge.	

YG MAJOR INCIDENT ACTION CARD:
Chaplaincy

Role:
Following instruction by the Hospital Incident Commander, the On-call Chaplain will be contacted by Switchboard.

	Action	**Completed By: Date & Time**
1	The role of the Chaplaincy Team during a major incident is as follows: • To assist patients, staff and relatives by providing reassurance and supportive listening. • To provide sacramental and pastoral care. • To assist the Hospital Command Centre (HCC), medical and mortuary staff in dealing with the needs of bereaved relatives. • To provide any practical help to staff as required. The Chaplaincy Centre will be kept as a calm quiet area for distressed or bereaved relatives and provide quiet space for staff. Chaplains will use it for sacramental and pastoral care and be available to medical or mortuary staff dealing with bereaved relatives. The Chaplaincy Centre should not become an extension to the main relatives centre.	
2	**Major Incident Declared:** The Chaplaincy Lead will: • Work under the instruction of the Relatives Officer to assess the level of chaplaincy cover needed. • Contact the out of hours Chaplain to cover the day to day commitments of the chaplaincy service. • Organise the call out of the required level of chaplaincy cover. • All Chaplains/assistants to congregate in the Chaplaincy Centre for the allocation of tasks. The Chaplaincy Team Leader will maintain a log of which chaplain is allocated to each area. • All Chaplains/assistants to report to the Chaplaincy Team Leader who will work under the direction of the relatives' officer.	

	• Lead Chaplain to liaise with wider faith communities to ensure pastoral support for specific faith groups e.g. Islamic, Jewish or Hindu communities for support if those involved belong to that community Ensure that chaplaincy cover is provided for the following: • Designated relatives area • Chaplaincy Centre • Emergency Department (if required) • Wards/Departments involved in dealing with casualties • Mortuary/viewing area (only at the request of the Relatives Officer, Police Identification Officer of Police Liaison Officer) • Staff rest area (at a later time when requested by the Hospital Incident Commander)	

YG MAJOR INCIDENT ACTION CARD:
Relatives Officer

Role:

The Information Officer will be responsible for identifying a member of staff to act as the Relatives Officer in the absence of the Senior Nurse for Clinical Governance – **this must be a Senior Nurse.**

	Action	**Completed By: Date & Time**
1	The **Relatives Officer** will be responsible for: • Setting up the Relatives Reception area in the Education Centre. • Identifying a clerk to record details. • Liaising with the Police Documentation Team and co-ordinating information. • Liaising with the Information Officer and providing updated information. • Co-ordinating Social Workers, Chaplains and Community Psychiatric Nurses. • Ensuring that MISPER forms are copied to the Information Room. • Ensuring beverages and refreshments are available for relatives and staff. • Organising regular updates for relatives in the Education Centre Lecture Theatre.	
2	**The layout of the Relatives Area is as follows:** **Front Desk:** Reception/Receiving Area **Dolbadarn Room:** Police Documentation Team **Harlech Room:** Relatives Officer **Penrhyn Room:** Relatives Waiting Area **Centre Manager's Office:** Breaking bad news area	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Health Records Officer

Role:

The allocated Health Records representative (Health Records Manager or another senior member of the Health Records Management Team) at Ysbyty Gwynedd will act as Health Records Major incident Manager.

Upon receipt of the standby / declared alert, inform the Hospital Command Centre of their readiness and await instruction.

	Action	**Completed By: Date & Time**
1	On receiving notification of a Major Incident they will co-ordinate the Health Records service and will • Call in 10 Health records staff which will include a supervisor and co-ordinator. o 4 staff will be assigned as "runners" led by 1 x supervisor / Co-ordinator o 4 staff will be assigned as desk based / remain in the library led by 1 supervisor / co-coordinator to take calls register patients and support as Loggists where necessary. Staff within these roles will rotated throughout the duration • A further 8 staff will remain on standby.	
2	Runners will be responsible for locating, retrieving, tracking and delivering case notes where required as soon as possible Desk based Library staff will be responsible for answering calls, registering patients, opening new notes, printing demographics and support as Loggists if required. **It is envisaged that these roles will rotate in a timely manner throughout the duration of the incident.**	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Locality Matron - Community Hospitals

Role:

When a Major Incident is declared, the Locality Matrons will be informed by the Area Incident Commander who will be located at the Area Control Centre.

	Action	**Completed By: Date & Time**
1	They will: • Support the NIC in establishing the community hospital bed status, co-ordinating discharges from Secondary Care, and co-ordinating patient transfers into appropriate community hospitals • Support the ACC in contacting off duty clinical and non-clinical staff, as required, to assist with the incident • Assist the ACC in identifying which staff can be redirected to Minor Injuries Units and / or other locations such as rest / reception centres to assist with minor injuries / walking wounded / chronic conditions management if required • Liaise with local services such as Social Services to assist with safe discharges from community hospitals • Provide staff briefings on the incident and any specific actions required to deliver the Area response. • Directly support and manage community hospital nursing teams throughout the incident, ensuring, on behalf of the Area Director, that their health, safety and welfare are being taken care of and that staff are being used in the most effective way. • Link in with the Home First Bureau to identify blockages in patient pathways and work together to further expedite discharges into the community, linking in with Tuag Adref, CRTs, LA and placements.	
2	• Provide remote management, on behalf of the Area Director, to any special arrangements activated, such as the Detection, Information and Monitoring plan.	

BLANK SHEET

This card is for the Locality Matron (West)

YG MAJOR INCIDENT ACTION CARD:
Locality Matron - District Nursing

Role:

When a Major Incident is declared, the Locality Matrons (District Nursing) will be informed by the Area Incident Commander who will be located at the Area Control Centre.

	Action	Completed By: Date & Time
1	They will: • Support the NIC in mobilising District Nursing staff to locations where casualties from the incident may require assistance with minor injuries/ wound care or chronic condition management • Support the NIC in identification of patients and changes to their vulnerability as a result of the incident • Support the NIC in reviewing and prioritising District Nursing activity, particularly in the area affected by the incident • Support the ACC in contacting off duty district nursing staff as required to assist with the incident • Liaise with local services such as Social Services to assist with safe discharges from community hospitals • Brief staff on the incident and any specific actions required to deliver the Area response • Directly support and manage Community / District Nursing teams throughout the incident, ensuring, on behalf of the Area Director that their health, safety and welfare are being taken care of and that staff are being used in the most effective way. • Link in with the Home First Bureau to identify blockages in patient pathways and work together to further expedite discharges into the community, linking in with Tuag Adref, CRTs, LA and placements.	
2	• Provide remote manage, on behalf of the Area Director, to any special arrangements activated, such as the Detection, Information and Monitoring plan.	

BLANK SHEET

This card is for the Radiology Duty Manager
During NORMAL working hours 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Radiology Duty Manager / Bleep 968 Holder (Card 1)

Role:
Inform the Senior of the Day of the situation and Radiology Services Manager.

	Action	**Completed By: Date & Time**
1	The Duty Manager/bleep 968 should then proceed to the Hospital Command Centre (HCC) with Action Card 3 (METHANE).	
2	On return from the Hospital Command Centre (HCC), Action Card 3 (METHANE) should be passed onto the Senior of the Day.	
3	Consider sending some staff home to relieve the remaining staff later in the day.	
4	Continue to monitor the situation and re-prioritise as necessary.	
5	Keep the Senior of the Day informed of any new information.	

BLANK SHEET

This card is for the Senior of the Day
During NORMAL working hours 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Radiology Senior of the Day (Card 2)

Role:
Senior of the Day (SOD) to be available at all times on SOD desk 842204 / 850907 or Bleep 095. (SOD to take dental bleep during major incident).

	Action	**Completed By: Date & Time**
1	Use a runner to hand out the departmental action cards to ED X-ray, CT Radiographers, Darkroom Staff, Nurses and Helpers, Office Staff, X-ray Porters, Main X-ray Radiographers, Radiologists and Secretaries.	
2	Use the designated runner to communicate between the modalities and to pass on any information to avoid the phone lines being tied up.	
3	Inform all modalities including St David's U/S.	
4	Inform the Clinical Director/Lead Radiologist of the situation and give them their Action Card.	
5	Work alongside the Duty Manager, keeping them informed of any new information.	
6	Organise Radiographers from the main department to go to Theatre when the calls come in, with the potential for all 3 IMI's and the mini c-arm being available for multiple theatres.	

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Radiology METHANE Report (Card 3)

Major Incident (Declared or Standby)	
Exact Location of Incident	
Type of Incident	
Hazards present at Site	
Access to the Incident Site	
Number of Casualties	
Emergency Services Currently at Site	

BLANK SHEET

This card is for the Radiographer
Working OUT OF HOURS 17.00 - 09.00, Monday to Friday and Sat/Sun

YG MAJOR INCIDENT ACTION CARD:
Out of Hours Radiographer (Card 4)

Role:

The Radiographer who takes the phone call on 842190 / 850907 or bleep 095 will receive the information “Major Incident Plan has been put into action – please proceed to the Hospital Command Centre”

	Action	**Completed By: Date & Time**
1	This Radiographer then informs their colleagues who are on duty of this information and gives them Action Card 5.	
2	One Radiographer then proceeds to the Hospital Command Centre with Action Card 3 (METHANE).	
3	The remaining colleagues follow the instructions on Action Card 5.	
4	The Radiographer returns from the Hospital Command Centre and disseminates the information to colleagues.	

BLANK SHEET

This card is for the Radiographer
Working OUT OF HOURS 17.00 - 09.00, Monday to Friday and Sat/Sun

YG MAJOR INCIDENT ACTION CARD:
Out of Hours Radiographer (Card 5)

Role:

Locate staff telephone numbers, which are located in the Major Incident Folder in ED X-ray. Also use the Radiology WhatsApp group to inform staff (General X-ray and CT Scan Groups).

	Action	**Completed By: Date & Time**
1	Telephone the Head of Service which is the first number on the list. If no answer, ring the next number on the list, which is in order.	
2	Inform them of the details you have been given.	
3	Give this person the following information – on call CT Radiographer, on call Radiologist, names of staff who are already on duty and the names of staff due to work in the next 24 hours.	
4	Also inform them of the current workload.	
5	Once you have contacted a member of staff and informed them of the incident do not phone anyone else, to prevent blocking any urgent incoming calls.	
6	Proceed with Action Card 6.	

BLANK SHEET

This card is for the Radiographer
Working OUT OF HOURS 17.00 - 09.00, Monday to Friday and Sat/Sun

YG MAJOR INCIDENT ACTION CARD:
Out of Hours Radiographer (Card 6)

Role:

Return inpatients to the wards safely.

	Action	**Completed By: Date & Time**
1	Switch on all X-ray rooms in the main department and all processing stations and computers.	
2	Turn on both CT Scanners and follow the check-up procedures.	
3	Prepare ED X-ray ensuring enough stock is available.	
4	Ensure all the mobiles are put on charge.	
5	Fetch another mobile to ED X-ray so there is more than one available. Ensure it is not causing an obstruction on the corridor.	
6	Open Ultrasound 1, key is located in ED X-ray.	
7	If time allows, ensure sufficient provisions for staff beverages etc.	
8	Await further instruction.	

BLANK SHEET

This card is for the Radiologist
Working OUT OF HOURS 17.00 - 09.00, Monday to Friday and Sat/Sun

YG MAJOR INCIDENT ACTION CARD:
Radiologist On-call (Card 7)

Role:

Assess the situation, details will have been recorded by the Radiographers on Action Card 3.

	Action	**Completed By: Date & Time**
1	Consider calling in extra Radiologists, according to the workload demands.	

BLANK SHEET

This card is for the Radiographers
Working 09.00 - 17.00 Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
ED Radiographers (Card 8)

Role:
Complete the imaging of patients already in the department and return them to clinic/ED.

	Action	**Completed By: Date & Time**
1	The SOD should ensure that there are 4 Radiographers in ED To deal with Majors and Resus.	
2	Ensure the mobile is on charge and fetch another one around so that there is more than one available.	
3	Ensure rooms are fully stocked.	

BLANK SHEET

This card is for the Radiographers
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
General Radiographers (Card 9)

Role:
Complete the imaging of any patients in the department.

	Action	**Completed By: Date & Time**
1	Room 5 needs to complete imaging the inpatients already in the department and arrange for their prompt return. DO NOT send for any further inpatients until the situation is resolved or there is an extremely urgent inpatient that needs imaging.	
2	Ensure rooms are fully stocked.	
3	All minor injury patients can be fetched around from ED X-ray to main X-ray to be imaged, utilising all of the rooms.	
4	One mobile machine should remain in the department for any urgent inpatients, ensure this is on charge.	

BLANK SHEET

This card is for the CT/MRI Radiographers
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
CT/MRI Radiographer (Card 10)

Role:
Ensure the cancellation of all OPD appointments on both CT scanners and the MRI scanner

	Action	**Completed By: Date & Time**
1	Return any inpatients to the wards.	
2	MRI staff to make themselves available to staff the CT Scanners/general X-ray. Where possible 3 Radiographers and an assistant on each scanner.	
3	Ensure the scanners and pumps are ready to go and there is an adequate supply of stock.	
4	Any extra CT/MRI staff could be considered to be sent home to return later to relieve staff if the incident is prolonged.	
5	Critically ill patients should be scanned on CT1 ideally and the less critical on CT2. In the event of 2 critically ill patients, the one patient to each scanner.	
6	Inform the CT Radiologist who MUST base themselves in CT1 for easy access by the Trauma Teams.	
7	For image transfers via IEP, contact 842147 (in hours) 850906 (16.30 – 08.00).	

BLANK SHEET

This card is for Darkroom Staff
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Darkroom Staff (Card 11)

Role:

Ensure the ED burner, located in the X-ray main office, is ready to use and is stocked with discs.

	Action	**Completed By: Date & Time**
1	In the event of Synapse issues, ensure the printer is switched on and ready to use, with an adequate stock of film.	
2	Be available on 4564 for image transfers via IEP.	
3	For image transfers via IEP, contact 842147 (in hours) 850906 (16.30 – 08.00).	

BLANK SHEET

This card is for Nursing Staff and Assistants
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Nursing Staff and Assistants (Card 12)

Role:
Ensure there is an adequate supply of linen, blue rolls, PPE etc in all rooms.

	Action	**Completed By: Date & Time**
1	If necessary, go to Linen Room to replenish stock.	
2	An Assistant should go to ED X-ray to assist with patients.	
3	Inform the SOD who has gone where.	
4	Assistants can switch on the Ultrasound Rooms and ensure they are stocked and ready to use.	

BLANK SHEET

This card is for Administration staff
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Administration (Card 13)

Role:
A member of the Administration Team will be based in ED X-ray to book in the patients.

	Action	**Completed By: Date & Time**
1	The main reception needs two members of staff to deal with any enquiries and to turn patients away.	
2	All request forms from the cancelled patients need to be kept to one side with a contact number to rebook.	
3	One member of the Administration Team should go to CT/MRI to assist with the cancelling of lists and the booking in of emergency patients.	

BLANK SHEET

This card is for the X-ray Porters
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
X-ray Porters (Card 14)

Role:

Remove any inpatients in the department to the wards promptly.

	Action	**Completed By: Date & Time**
1	One Porter should attend the Porter's Lodge to pick up a radio and return to X-ray.	
2	Follow any instructions given over the radio.	

BLANK SHEET

This card is for the Radiologist
Working 09.00 - 17.00, Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Radiologists (Card 15)

Role:
The Radiologist will assess the situation.

	Action	**Completed By: Date & Time**
1	Allocate the resources to the appropriate clinical areas.	
2	Consider sending some staff home to return later to relieve another Radiologist.	
3	All ED images will be put into the hot reporting folder for immediate reporting to allow the prompt discharge of patients.	
4	The CT Radiologist needs to be based on CT1 to be easily accessible for the discussion of radiology requests from the Trauma Teams.	

BLANK SHEET

This card is for the Secretaries
Working 09.00 - 17.00: Monday to Friday

YG MAJOR INCIDENT ACTION CARD:
Radiology Secretaries (Card 16)

Role:

Liaise with the Radiologists to determine who will be reporting CT and plain films and determine who needs a typist.

	Action	**Completed By: Date & Time**
1	Ensure the reports are typed and are readily available for ED as soon as possible.	

BLANK SHEET

This card is for the 1st On-call Consultant Anaesthetist (642)

YG MAJOR INCIDENT ACTION CARD: On-call Consultant Anaesthetist

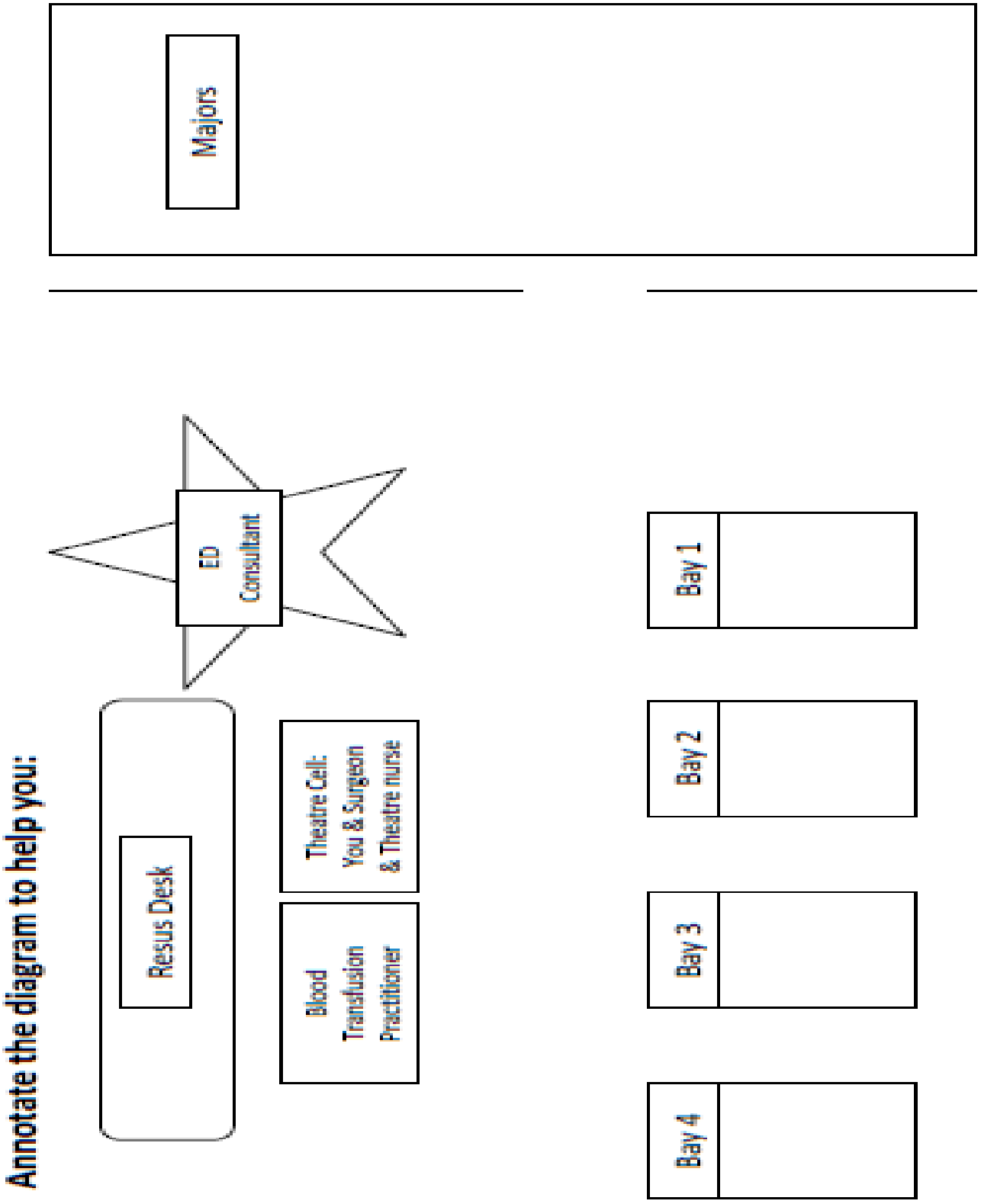
	Action	**Completed By: Date & Time**
1.	Use the WhatsApp group 'YG Anaes Major Incident'. Send "MAJOR INCIDENT DECLARED. RSVP with your name, role and availability. GO TO YG THEATRE RECEPTION"	
2.	Call a second Consultant. State "Major Incident Declared". Out of hours – Call anaesthetic admin OR appoint second Consultant as admin person and tell them to follow action card for Anaesthetic Admin. Normal working hours - inform Anaesthetic Admin to follow the Anaesthetic Admin Action Card.	
3.	Pick up the major incident action card folder from the anaesthetic dept admin office. Take your card and take the folder to theatre reception. Leave the folder in theatre reception. Don the 'On-call Consultant Anaesthetist' tabard from the major incident box.	
4.	Make your way to the Emergency Department and meet with ED Consultant and Clinical Site Manager for briefing.	
5.	Allocate Anaesthetist and ODP's to Resus bays and assist to make bays ready for major trauma patients. See Resus Anaesthetist Action Card.	
6.	Aim for minimum 1 ED Doc, 1 ED Nurse, 1 Anaesthetist, 1 ODP per bay. Double up Anaesthetists if possible.	
7.	Contact Co-ordinating Anaesthetist regularly to give situation reports on 850153. Give them one contact number to call you on.	
8.	Co-locate with Surgeon and Theatre Nurse to form Theatre Cell in Resus.	
9.	Act as the Anaesthetic Co-ordinator for ED: Allocate, supervise and support all anaesthetists in Resus Support ED Consultant and Surgeon with triage and decision making (e.g. which patients to CT / Theatre first).	
10.	Maintain flow of patients. Defer RSI and procedures until patient in theatre if safe to do so.	

11.	If moving patients to Theatre / ICU, CONFIRM DESTINATION with Co-ordinating Anaesthetist on 850153.	
12.	Contact Co-ordinating Anaesthetist on 850153 if any of the following needed: Booking a case for theatre Transfer Team Mobile Medical Team Extra staff for Resus Cover for breaks	
13.	Document your briefs, assessments, interventions and decisions.	
14.	This is a difficult job. If you are getting fatigued, contact the Co-ordinating Anaesthetist on 850153 to arrange relief.	

NOTES:

- You are overall Lead for Anaesthetics until the Co-ordinating Anaesthetist has arrived. Then they are the Lead, but your focus will be more on supporting Triage, Resuscitation and decision making in Resus.
- All cases need to be booked by phone in Theatre Reception. Don't just send patients to Theatre. Make sure they can contact you easily.
- 551 / 063 RESPONSIBLE FOR OBS EMERGENCIES / Cardiac Arrest CALLS - Make sure they are doubled up / free to attend an emergency
- Wear your ID badge at all times

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		


Majors
Annotate the diagram to help you:
ED Consultant
Resus Desk
Theatre Cell: You & Surgeon & Theatre nurse
Blood Transfusion Practitioner
Bay 1
Bay 2
Bay 3
Bay 4

BLANK SHEET

This card is for the Resident On-Call Anaesthetist (Bleep 063)

YG MAJOR INCIDENT ACTION CARD:
Resident On-Call Anaesthetist (Bleep 063)

	Action	Completed By: Date & Time
1.	Contact Consultant On-call immediately and inform them of the incident.	
2.	If you are committed in theatres and unable to attend ED, inform the other on-call residents and the on-call Consultant. Contact the Co-ordinating Anaesthetist at theatre reception (850153) when you are free.	
3.	If you are available, go to the anaesthetic department and pick up one of the Resus anaesthetist action cards from the major incident folder in the admin office. Go to ED.	
4.	You remain the On-call Anaesthetist for emergency calls so try and double with another Anaesthetist as soon as possible.	
5.	If called for an anaesthetic emergency or cardiac arrest, you are responsible for attending or finding alternative cover.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

BLANK SHEET

This card is for the anyone appointed as Admin person by the On-call Anaesthetist
It can be: Anaesthetic Secretary, Trainee Nurse, SG or Consultant
2 people may be required initially

YG MAJOR INCIDENT ACTION CARD:
Anaesthetic Admin Person

	Action	Completed By: Date & Time
1.	IN HOURS - Do walk round of department offices and coffee rooms and send everyone to Theatre Reception. OUT OF HOURS - Go straight to Task 2.	
2.	Call people using off-duty call sheet (or CLW) starting with consultants, then staff grades / middle grades, then junior residents. Check the WhatsApp Major Incident Group and don't call those who have already RSVP'd. Make a note of who you called, who is coming and when. Send everyone to Theatre Reception. Once at least 4 consultants are attending, make your way to the Anaesthetic Department.	
3.	Open Anaesthetic Department and leave it unlocked.	
4.	Pick up all spare bleeps and take them to Theatre Reception. Allocate to runners.	
5.	Confirm that there is a consultant in ED and a co-ordinating consultant at theatre reception together with theatre co-ordinator.	
6.	Delegate someone to start the incident diary (kept in the Major Incident Box in Theatre Reception) and note all decisions and timings.	
7.	Help the Co-ordinating Anaesthetist in Theatre Reception: Continue to call in staff as necessary Log where all Anaesthetists are (on the white board) Send surplus people to Anaesthetic Department or home Keep people from crowding Theatre Reception and try to keep noise levels to a minimum Organise breaks if necessary Arrange next shifts including Co-ordinating Anaesthetist and yourself.	

Off Duty Callout

Use the callout sheet provided – target people near the hospital first, then people in the non-affected direction from the incident.

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for the first Anaesthetic Consultant to arrive At Theatre Reception (not On-call)

YG MAJOR INCIDENT ACTION CARD:
Co-ordinating Anaesthetist

	Action	Completed By: Date & Time
1	If you are the first Consultant Anaesthetist at Theatre Reception, then this card is yours until you handover the position	
	Stay at Theatre Reception and co-locate with Theatre and ODP Co-ordinators. Don tabard and start completing the Major Incident Whiteboard.	
2	Contact Anaesthetic / Theatres Admin for comms and admin support. Appoint one to take calls and manage the whiteboard. If possible, have one person dedicated for each phone.	
3	Appoint a scribe to keep the incident diary and log all comms and decisions. Consider voice recorder if available.	
4	Contact Consultant Anaesthetist On-call for situation report (sit rep) from ED.	
5	Contact Hospital Command Cell (HCC) on 03000 850823 and identify your position and contact number. Relay sit rep from ED.	
6	Log in staff as they arrive and ALLOCATE ROLES (attached card) *Pair Consultants with Trainees once enough staff available.*	
7	Contact ICU / HDU for bed status - put on Whiteboard.	
8	Contact ICU Consultant to confirm ICU MI Plan activated.	
9	Support the THEATRE LEADER in deciding which lists to stop and theatres to prepare.	
10	Keep Resus Anaesthetists with their patient when they come to theatre and send a fresh (reserve) team to Resus for new patients.	
11	Keep reserve teams in the Anaesthetic Department (Ext 842041 / 842042). Make sure they are contactable. Use them to give breaks.	
12	Send surplus staff home as soon as possible to prepare for next shift.	
13	As cases are booked for theatre, write them on the whiteboard and allocate to a theatre in order of priority and availability.	
14	Contact Anaesthetic Consultant in ED and ICU Consultant regularly for Sit Rep.	

15	Contact HCC on 03000 850823 every 30 minutes to give Sit Rep.	
16	Organise your own and admin replacement for end of shift.	

NOTES:

- The Theatre Leader is in overall charge of Theatres. Support them to make their decisions.
- The incident (from a theatre perspective) will be managed from Theatre Reception.
- Establish comms with the Anaesthetic Consultant / Theatre Cell in ED - preferably via one direct landline number.
- Consider keeping less senior staff (even novices) in elective cases where appropriate with one Consultant providing overall supervision for theatres and recovery.
- It will be busy and noisy. Try and keep people away - send them to the coffee room or Anaesthetic Department but make sure they stay contactable. Send surplus people home to prepare for the next shift.
- Ensure all patients (including normal CEPOD and trauma emergencies) are listed on whiteboard.
- Theatre Cell in ED (Consultant Anaesthetist, Surgeon and Theatre Nurse) will decide priority for theatres and will phone Theatre Reception to book bases. Actual order of cases may depend on theatre, X-ray and surgeon availability etc.
- If there is only 1 theatre free then save it for a potential time critical case that you may not yet know about.
- You need to call Resus Team (Theatre Cell) when theatre is ready to accept a patient.
- Keep Resus Anaesthetists with their patient when they come to theatre and send a fresh (reserve) team to Resus for new patients.
- 551 / 063 RESPONSIBLE FOR OBS EMERGENCIES / Cardiac Arrest CALLS - Make sure they are doubled up / free to attend an emergency.
- This is a difficult job and if you are getting fatigued, then arrange handover to another Consultant.

Hospital Command Centre 03000 850823

COMMUNICATIONS				
851038/9	**Recovery**	841831	**THEATRE**	
851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
#045 / 840044	**CT Scan**	850902	**6**	850952/3
850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**	

BLANK SHEET

Role	Name	Time
Immediate Roles – Allocate First		
Consultant On-call in Resus	(1st on call Consultant)	
Resus 1		
Resus 2		
Resus 3		
Resus 4		
ICU Resident 1	(the on-call ICU resident)	
Theatre Anaesthetists	**Keep the Resus Anaesthetics with their patient when they come to theatre and send a fresh (reserve) team to Resus.** **Consider keeping less senior staff in elective cases with one Consultant providing overall supervision for theatres and recovery.**	
Theatre / Recovery Consultant		
ICU Consultant (if not already on rota)	Choose an ICU Consultant who does regular ICU sessions)	
Reserve team 1 (e.g. for Resus)		
2nd ICU Consultant (to run auxiliary ICU)		
ICU Resident 2		
Reserve Team 2		
ED Majors Team		
Recovery / ICU Resident 3		
Reserve Team 3		
Secondary Roles		
Mobile Medical Team	Choose people with pre-hospital experience if possible	
Transfer Team 1		
Transfer Team 2		
Replacements for On-call Team		
Replacement for Co-ordinating Anaesthetist		
Staff for Next Shift		

BLANK SHEET

This card is for the Obstetric On-call Anaesthetist

YG MAJOR INCIDENT ACTION CARD:
Obstetric On-call Anaesthetist

	Action	Completed By: Date & Time
1	Your priorities are Obstetrics. You must remain free to attend obstetric emergencies.	
2	If not busy on Obstetrics, contact Co-ordinating Anaesthetist (Theatre Reception / 8861) or if they are not available, contact the On-call Consultant Anaesthetist in ED.	
3	If they are busy, go to ED Resus and help prepare for major trauma patients (see Resus Anaesthetist Action Card).	
4	Ensure you are doubled up asap and free to attend obstetric emergencies.	
5	If not required in Resus, go to ITU and help review patients suitable for discharge and prepare for new admissions.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

BLANK SHEET

This card is for all Anaesthetists on a theatre list (inc Theatre 2 Trauma list)

YG MAJOR INCIDENT ACTION CARD:
All Anaesthetists on a Theatre List

	Action	Completed By: Date & Time
1	If you are doubled up, then if appropriate, the more junior person should stay and the more senior person should contact the Co-ordinating Anaesthetist in Theatre Reception.	
2	Contact Theatre Lead (Theatre Reception / 850960) and Co-ordinating anaesthetist (850153) when safe to do so.	
3	Finish case you are on and do not send for any more unless Major Incident Stand Down called or Theatre Lead allows list to continue.	
4	Discuss with Surgeon which cases can be cancelled or postponed for next 24 hours and which need to be done today.	
5	Report to Theatre Reception if your list is finished.	
6	Help to prepare theatres to receive patients.	

COMMUNICATIONS				
851038/9	**Recovery**	841831	**THEATRE**	
851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
#045 / 840044	**CT Scan**	850902	**6**	850952/3
850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
851432	**TTL Stoke or 01782 674444**	07623 975903		

Theatre Reception:	**Theatre booking**	**850961**
	Co-ordinating Anaesthetist / ODP	**850153 / 841830**
	Theatre Lead	**850960**

BLANK SHEET

This card is for all Anaesthetists who attend ED Resus. Only go to ED Resus if your Action Card or the Co-ordinating Anaesthetist tell you. Otherwise go to Theatre Reception

YG MAJOR INCIDENT ACTION CARD:
Resus Anaesthetists (8 Cards)

	Action	Completed By: Date & Time
1	Go to ED Resus, report to On-call Consultant Anaesthetist and prepare bays for major trauma patients. Use the trauma team stickers in Resus to identify yourself as anaesthetist.	
2	Prepare drugs for RSI, Analgesia, TXA, Antibiotics, Emergency drugs. You may need to take drugs from Theatres.	
3	Check equipment: Airway, Ventilator, IV/IO Access, Monitors, Syringe Drivers.	
4	Aim for at least 1 Anaesthetist, 1 ODP, 1 ED Dr and 1 Nurse per bay. If possible double up - 1 Consultant with 1 Trainee.	
5	Be prepared to act as Team leader for your bay if required.	
6	Prepare blood bottles and forms for X-match, ROTEM, ABG, FBC.	
7	Use hospital D Number, DoB and name on all forms for blood request, CT, theatres etc. Unknown patients will be given an allocated name and estimated DoB on booking in.	
8	Prepare for the unexpected. Pre-alert information may be scanty, absent or wrong.	
9	Assist with all critical care interventions, access, analgesia.	
10	Speak to On-call Consultant Anaesthetist in Resus if you need support or advice.	
11	Stay with your patient unless you are retasked.	
12	If patient needs blood: liaise with Blood Transfusion Practitioner in ED (or call lab on 842371 if they are not there). Use sparingly and use O+ve for males.	
13	If patient needs RSI: inform On-call Consultant Anaesthetist in Resus. If patient going to theatre, consider deferring RSI until in theatre. Always use the RSI checklist.	

14	If patient needs surgery, inform the Theatres Cell / Consultant Anaesthetist in ED Resus (or call Theatre Reception on 850960 / 850153).	
15	Don't take patient to theatre until theatres have confirmed they are ready.	
16	Stay with your patient if they go to theatre.	
17	Ensure you have signed in and document you assessment, interventions and decisions clearly in the patient's notes.	
18	If patient needs transfer to another hospital, contact the Co-ordinating Anaesthetist on 850153.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for the Consultant Intensivist

YG MAJOR INCIDENT ACTION CARD:
On-call Intensive Care Consultant

	Action	Completed By: Date & Time
1	Report to Theatre Reception with ICU Nursing Co-ordinator for report from Co-ordinating Anaesthetist (850153).	
2	Co-ordinate from the Cybi seminar room. Review critical care bed state with ICU Nursing Co-ordinator.	
3	Decide with Co-ordinating Anaesthetist: Need to escalate ICU capacity ICU Staffing needs Communicate with Hospital Incident Commander 03000 840490 regarding transfer of existing Enlli patients.	
4	Decide on movements of existing and new Level 3 patients across ICU/HDU (benefit of cohorting new patients on ICU vs time/personnel loss in movements and overwhelming one location with new patients). Identify critical care DTOC's for discharge to ward Identify critical care patients for outlying to Enlli (must be Level 2 patients or DTOC's)	
5	Allocate available staff to clinical areas: • Ensure ICU, HDU and Enlli each have Consultant and Resident Anaesthetist • Allocate MET call / cardiac arrest response to a team that have capacity / can respond appropriately. • Confirm nursing needs with ICU Nursing Co-ordinator	
6	Ongoing role: Once Ward teams allocated, do not become involved in direct patient care Accept and discharge incident related and non-incident patients, informing Co-ordinating Anaesthetist If inadequate critical care capacity develops, inform Co-ordinating Anaesthetist and hospital command centre (HCC). Support critical care ward teams equipment and skills requirements Hourly sit-reps to Co-ordinating Anaesthetist and Major Incident Commander	

	Check with Co-ordinating Anaesthetist for ongoing staffing needs on future shifts	
7	Discuss with Co-ordinating Anaesthetist regarding bed de-escalation.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for Anaesthetists (Consultants, Junior Teams) covering or allocated to clinical care

YG MAJOR INCIDENT ACTION CARD:
Critical Care Doctor Teams

Role:

Ensure your team has a Consultant and Junior Anaesthetist / Intensivist.

	Action	**Completed By: Date & Time**
1	Ensure your team has a Consultant and Junior Anaesthetist / Intensivist.	
2	Once allocated to role, report to Seminar Room for tasking from ICU On-call Consultant and ITU Nursing Co-ordinator who will allocate a critical care ward (Cybi, HDU, Enlli).	
3	Communicate with ICU On-Call Consultant and Nursing Co-ordinator and ward Nurse in Charge only.	
4	Do not leave your ward without informing the Consultant On-call.	
5	Roles: • Ensure adequate equipment / monitoring and drugs available in your area • Communicate shortages with ICU Nursing Co-ordinator • Ensure daily ward review done on existing patients if able • Receive handover from transferring ED / Theatre Teams • Ensure secondary and tertiary survey are completed • Arrange investigations and ongoing care of new and existing patients • Alert ITU Consultant On-call of all patients ready for discharge • Respond to arrest calls only if role allocated to you	
6	Alert ITU Consultant On-call if extra resource or staffing relief required	

Hospital Command Centre 03000 850823

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

YG MAJOR INCIDENT ACTION CARD:
ICU Nursing Co-ordinator

	Action	Completed By: Date & Time
1	Allocate a senior member of staff to assume role of Nurse in Charge of existing ICU/HDU patients.	
2	Base yourself in the Seminar Room on Cybi Ward - you will remain supernumerary throughout the incident.	
3	Report with the on call ICU consultant to the Co-ordinating Anaesthetist (Theatre Reception) and inform them of bed / staff status and obtain details of the incident.	
4	Instruct a member of your team to call in sufficient staff, using dedicated 'WhatsApp' group for the immediate situation and place others on standby.	
5	Liaise with the ICU Resident or Consultant if available, of patients that may be safely transferred out of the unit.	
6	Arrange for the transfer of these patients liaising with Bed Managers.	
7	Allocate one staff member to act as a runner between ED and ICU, instruct the nurse to report back to you at regular intervals.	
8	Ascertain bed status in other North Wales and North West ICU's.	
9	Discuss with Co-ordinating Anaesthetist (Theatre Reception), Hospital Incident Commander (03000 840490), ICU Anaesthetist about whether to activate the Auxiliary ICU Plan.	
10	Ensure that arrangements are made for rest / meal breaks and relief for your staff.	
11	Co-ordinate shift change as appropriate.	
12	Ensure all staff report to you before leaving the hospital to be debriefed and thanked.	

Hospital Command Centre 03000 850823

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for Nurse in Charge of each Critical Care Ward Area

YG MAJOR INCIDENT ACTION CARD:
Critical Care Ward Nurse in Charge

Role:

Ensure Consultant in Charge of your ward knows you are the Nurse in Charge.

	Action	Completed By: Date & Time
1	Ensure Consultant in Charge of your ward knows you are the Nurse in Charge.	
2	Obtain Bleep numbers from your Medical Team.	
3	Ensure adequate equipment / ventilator / monitoring / drugs / documentation for each bed space.	
4	Create a status board for your area.	
5	Contact ICU Nursing Co-ordinator with hourly updates.	
6	Ensure adequate rest periods.	
7	Ensure all staff report to ICU Nursing Co-ordinator prior to leaving the hospital.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

BLANK SHEET

YG MAJOR INCIDENT ACTION CARD:
Critical Care Nurse

	Action	Completed By: Date & Time
1	Report to the Nursing Co-ordinator in the Seminar Room on Cybi.	
2	Be directed by the Nursing Co-ordinator, do not undertake any tasks which have not been allocated to you by the Co-ordinating Nurse.	
3	Once allocated to specific critical care ward, do not undertake any tasks not allocated to you by your Ward Nurse in Charge.	
4	If allocated to Enlli - obtain list of items needed from Major Incident Box. Refer to Enlli as 'Enlli Ward' at all times - not 'HDU'.	
5	If you need additional support or rest at any time, inform Ward Nurse in Charge.	
6	If allocated to another area of the hospital i.e. ED, report to the Nursing Co-ordinator on ICU at regular intervals. Do not leave the hospital without the knowledge of the Co-ordinating Nurse.	
7	Wait in the coffee room on Cybi if you are not needed immediately.	
8	Inform the Nursing Co-ordinator before you leave the hospital so you can be debriefed and thanked.	

Hospital Command Centre 03000 850823

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for the ICU Ward Clerk

YG MAJOR INCIDENT ACTION CARD:
ICU Ward Clerk

Role:
Report to ITU Nursing Co-ordinator in the ITU Seminar Room.

	Action	**Completed By: Date & Time**
1	Report to ICU Nursing Co-ordinator in the ICU Seminar Room.	
2	Do not leave critical care without informing the ICU Nursing Co-ordinator.	
3	Refer all calls regarding bed states or incident management to ICU Nursing Co-ordinator.	
4	Ongoing Roles: • Update WPAS for all admissions, discharges and transfers • Ensure notes are in order for all new admissions and discharges • Contact Ward Nurse in Charge for all patient enquiries	
5	Report to ICU Nursing Co-ordinator prior to leaving the hospital.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

BLANK SHEET

This card is for the Theatre Co-ordinator (Bleep 019/611)

YG MAJOR INCIDENT ACTION CARD:
Theatre Co-ordinator

	Action	Completed By: Date & Time
1	Base yourself in the main theatre reception with the co-ordinating anaesthetist and coordinating ODP (Ext 850153). You oversee theatres. Your main role will be coordinating which patient will go to which theatre and ensuring theatres are adequately resourced.	
2a	**Out of normal working hours:** Using the staff contact number list (kept in a sealed envelope in theatre recovery) call in off duty staff. Start with the Team Leaders. Aim to staff 2 general and 2 orthopaedic theatres to begin with. Tell them to report to theatre reception and to carry their ID badge. **Delegate this task as soon as possible.**	
2b	**In hours:** Stop all elective surgery that has not already commenced. Send a runner to every theatre informing them of the major incident. Ask for all team leaders to report to you with an estimated finish time.	
3	Open the major incident box (store room next to reception) and hand out the tabards, logbooks and whiteboard pens.	
4	Hand out the action cards for: Telephone operators, Scribe/loggist Runners.	
5	Delegate a Recovery nurse or ODP to follow the Action Card: 'Theatre Co-ordinator in ED'	
6	Initiate preparation of 2 x General and 2 x Trauma theatres for cases until further instructions are given. Prioritise theatres 4,6 (general) and 2,12 (ortho). Be prepared to open more theatres if necessary.	
7	Contact CSSD to inform them of potential requirements	
8	Contact on duty ODO. Ensure they are following their action card.	

9	Ensure that relief breaks are organised for all scrub/circulating or recovery staff.	
10	If staff are not required, then send them home to ensure there are rested staff for further shifts or later routine work.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for anyone who has been designated by the Theatre coordinator (ODP / recovery nurse)

YG MAJOR INCIDENT ACTION CARD:
Theatre Co-ordinator in ED

	Action	Completed By: Date & Time
1	Locate yourself in ED Resus with the on-call anaesthetic consultant and surgeon. Don the appropriate tabard. Your main role is to communicate between Theatre reception and ED. It is important that you carry your hospital ID badge and bleep throughout the incident.	
2	You will liaise with the theatre coordinator / coordinating ODP/ coordinating anaesthetist at theatre reception on 850153. Make sure they have one contact number for you. Answer promptly if called.	
3	You are responsible for informing the team in ED when the theatre department is ready to take patient.	
4	You will inform theatre reception when a patient is on the way to theatre reception.	
5	You will liaise with the theatre coordinator in theatre reception if there is any change in requirements in your area of if you need relief.	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

BLANK SHEET

This card is for the first (not on-call) senior ODP to arrive at Theatre eception

YG MAJOR INCIDENT ACTION CARD:
Co-ordinating ODP

	Action	Completed By: Date & Time
1	This will be the first senior ODP to arrive in theatre (not 1st on-call) Base yourself in Theatre Reception (ext 841830) with the theatre co-ordinator and the coordinating anaesthetist. Your main role is to deploy ODP staff during the incident as required. Stay supernumerary and do not take a clinical role unless necessary. The Obstetrics ODP needs to always be available for obs emergencies.	
2a	Out of normal working hours: Using the staff contact number list (kept in a sealed envelope in theatre recovery) call in off duty staff. Start with the Seniors. Aim for 4 ODPs in ED, 1 for ICU and 4 for theatres to start with. Tell them to report to theatre reception and to carry their ID badge. Delegate this task as soon as possible.	
2b	In hours: Stop all elective surgery that has not already commenced. Send 4 ODPs to ED – take from elective lists if necessary. Inform remaining ODPs to assist in any remaining elective cases, and prepare all theatres for potential cases (prioritise 2,4,6,12) Inform all ODPs to report to you when duties complete.	
3	Ensure that arrangements are made for rest/meal breaks and relief.	
4	When the extent of the incident has become clear those not needed immediately should be sent home to rest as soon as possible and remain contactable.	
5	Ensure all staff report to you before leaving, to be debriefed and thanked.	
6	Don’t forget relief will be needed for you.	

Hospital Command Centre 03000 850823

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

This card is for the Operating Department Practitioners

YG MAJOR INCIDENT ACTION CARD:
On-Call Operating Department Practitioners (ODP)

	Action	Completed By: Date & Time
1	Both resident ODPs will be contacted by Switchboard informing them of 'Major Incident Standby – please be prepared to assist OR Major Incident Declared – please report to theatres'	
2	If the call is Major Incident Standby – The 1st on-call ODP is to: Contact the 2nd on call ODP and inform them of the standby. Locate the action cards, tabards and call out lists Wait for further information.	
3	If the call is Major Incident declared – The 1st on call ODP (017) is to report to on call consultant anaesthetist and theatre coordinator in ED Resus (bear in mind you may be there before them). Prepare RSI drugs (Ketamine, Fentanyl, Rocuronium) and Tranexamic Acid Prepare to assist with RSI and other interventions Call Theatre reception (841830) if you need more ODPs in ED / help / you need a break. Obstetric ODP is to: Contact the 2nd on-call ODP and tell them to report to theatre reception immediately. If you are free, help to set up theatres 2,4,6 and 12 ready to receive patients. Then report to Theatre reception. Act as ODP coordinator until a senior ODP arrives to take over. Always stay available for obstetric emergencies.	
4	Remember to wear your hospital ID badge at all times.	

BLANK SHEET

This card is for the Theatres Telephone Operator (anyone can fill this role)

YG MAJOR INCIDENT ACTION CARD:
Theatres Telephone Operator (x2)

	Action	Completed By: Date & Time
1	Locate yourselves in Theatre Reception. Answer the phone immediately every time it rings.	
2	Make sure you have enough of the Telephone Operator sheets and working pens.	
3	Write down all information and messages received. Repeat the message back to the sender to ensure you have accurately recorded it.	
4	Report all messages to the coordinating team but only interrupt them if the message is urgent.	
5	Try to keep noise levels to a minimum	
6	Report to the theatre coordinator if: In need of relief If required to move out of the area Before leaving the hospital	

COMMUNICATIONS					
ED Resus	851038/9	**Recovery**	841831	**THEATRE**	
ED Majors	851033 / 851034	**Enlli Ward**	841351	**2**	850945/6
ED Reception	851010	**Anaesthetic Dept**	842041 / 842042	**3**	850947/8
BLOOD BANK	842371	**Hospital Command Centre (HCC)**	850823	**4**	850949
Haematology	#043 / 840044	**Executive Seminar Room**	851422	**5**	850950/1
Biochemistry	#045 / 840044	**CT Scan**	850902	**6**	850952/3
ICU (CYBI)	850829 / 841341	**NWTS**	08000 848382	**7**	850954/5
HDU	841310	**EMRTS / ACCTS Critical Care Hub**	0300 1232301	12	850942
PACU	851432	**TTL Stoke or 01782 674444**	07623 975903		
Theatre Reception:	**Theatre booking** **Co-ordinating Anaesthetist / ODP** **Theatre Lead**		**850961** **850153 / 841830** **850960**		

BLANK SHEET

This card is for the Operating Department Orderly (ODO)

YG MAJOR INCIDENT ACTION CARD:
Operating Department Orderly (ODO)

	Action	**Completed By: Date & Time**
1	You will be alerted by the Theatre Coordinator that a major incident has been declared.	
2	Locate the list of ODOs and their home telephone numbers and begin calling them in to the hospital starting with the more senior staff member.	
3	You will assist the nursing staff in opening and beginning to prepare theatres.	
4	You will report to the Co-ordinating ODP in Theatre Reception (841830), to inform them who you have managed to contact and who is coming in.	
5	You will then be directed by the Co-ordinating ODP as to where you should go.	
6	Always inform the Co-ordinating ODP when your job is complete or if you need additional support or rest.	
7	Return to the ODO room if you are not working.	
8	Report to the Co-ordinating ODP before you leave the hospital to be debriefed and thanked.	
9	Remember to wear your hospital ID badge at all times.	

BLANK SHEET

This card is for the Theatre staff not on duty

YG MAJOR INCIDENT ACTION CARD:
Theatre Staff NOT ON DUTY

	Action	**Completed By: Date & Time**
1	You will be contacted by a member of theatre staff stating a Major Incident has been declared and requesting that you attend the hospital.	
2	You will: • Proceed to theatres, entering the hospital by the Main Entrance • You will report to the Theatre Coordinator who will be in Theatre Reception • You will go to the specified area and prepare as directed. Do not leave this area without informing the Theatre Coordinator.	
3	Follow the Action Card for Runners for rest of the instructions. If you have nothing to do: report to theatre reception and wait in the coffee room for further instructions. Remember to wear your hospital ID badge at all times.	

BLANK SHEET

This card is for the Theatre Runners

YG MAJOR INCIDENT ACTION CARD:
Theatre Runners

	Action	Completed By: Date & Time
1	You will be informed by the Theatre Coordinator that a major incident has been declared.	
2	Start preparing theatres as directed by the Theatre Coordinator.	
3	You must inform the Theatre Coordinator of any additional need that you may have in the area in which you are working, or if staff requires rest.	
4	You must inform the Theatre Coordinator if you need to leave the area in which you have been placed.	
5	If you are not allocated to an area you should wait in the coffee room so you can be located quickly if needed.	
6	You must inform the Person in Charge of Theatre before you leave the hospital so you can be debriefed and thanked.	
7	Remember to wear your hospital ID badge at all times.	

BLANK SHEET

Appendix 1

Handover Procedure	
Role:	**HIC MIC NIC (please circle)**
Name:	
Title:	
Date:	
Time:	
Signature:	
Handed Over To:	
Name:	
Title:	
Date:	
Time:	
Signature:	

Reason For Handover:

BLANK SHEET

Appendix 2

JESIP
Working Together – Saving Lives

Co-locate
Co-locate with commanders as soon as practicably possible at a single, safe and easily identified location near to the scene.

Communicate
Communicate clearly using plain English.

Co-ordinate
Co-ordinate by agreeing the lead service. Identify priorities, resources and capabilities for an effective response, including the timing of further meetings.

Jointly understand risk
Jointly understand risk by sharing information about the likelihood and potential impact of threats and hazards to agree potential control measures.

Shared situational awareness
Shared Situational Awareness established by using METHANE and the Joint Decision Model.

Joint Decision Model:

